NIHR | National Institute for Health and Care Research

Summary

Reference Number	NIHR306437
Lead Applicant	Dr Lotte Elton
Research Title	The UNITE study: UNderstanding and Improving how blood TEst uncertainty, overuse and overdiagnosis are discussed in primary care – a participatory ethnographic study to improve clinical practice
Plain English Summary	**Background** Communicating effectively about blood testing is important for ensuring patient safety and involvement in healthcare. Blood tests are useful, but do not always improve patient care. Sometimes they can cause harm: for example, borderline test results may worry patients or lead to unnecessary further tests. This is important because blood tests have become increasingly common: general practice (GP) testing rates have tripled since 2000. Previous research has shown that patients often feel unsure about why blood tests are done and what results mean. GP staff do not always clearly explain the reasons for testing and the limitations of blood tests, and patients report not having enough opportunity to speak to staff about testing. This means that GP staff and patients may have different expectations and understandings of blood tests. Understanding what happens in consultations about blood tests is important for addressing the rise in testing and ensuring tests are used wisely. **Aims** This project will explore how blood tests are discussed between patients and GP staff, aiming to improve communication about the limitations and risks of blood tests. The goal is for patients to be more involved in decisions about blood testing and understand better what blood tests mean for their health. **Research design** The project has four parts: 1. *Evidence search:* I will search and review existing research on how blood tests are discussed in general practices. This will help to understand what is already known and what is missing. 2. *Observation at GP practices:* I will spend several months watching conversations about blood tests at two different general practices. I will make recordings of patient consultations with GP staff (including GPs, nurses, pharmacists and phlebotomists) to look at how blood tests are discussed. 3. *Workshops with GP staff:* I will show short recordings of blood test discussions to staff. They will consider how blood tests are discussed with patients and think about how communication can be improved. 4. *Theatre workshops with the public:* Professional actors and public contributors will create a short play based on the research. This will be performed to members of the public as part of "Forum Theatre" workshops. Workshop attendees will be invited to make changes to the play to improve communication about blood tests. Public contributors will help to develop the play into two tools to improve blood test communication: a. A workshop for general practice staff. b. An online education resource for patients. **Patient and Public Involvement (PPI)** People who have recently had blood tests will be involved throughout the project. Nineteen public contributors with diverse ages, ethnic backgrounds, and health experiences helped to shape the project over three workshops. They described many issues with communication about blood tests and felt strongly that this research was important. A PPI panel will guide the research to make sure the project responds to the needs and preferences of people having blood tests. The panel will meet three times a year to review project plans at each stage and provide input on data analysis. The panel will also write public summaries of the research. Panel members will be paid for their contributions. They will receive funding for training

	and attending conferences to share the research. Two lay people will co-chair a separate Project Advisory Group to steer the project. **Sharing the research** I will share the research with local community groups, patients and GP practice staff through social media, blogs, a podcast, and a public event. The online resource for patients will be made available on a project website. I will use the workshop tool created from the play to train GP staff and trainees on better blood test communication.

Our purpose for collecting this data is to communicate with you about your application and have the necessary information to evaluate you for a grant. The data we collect here is collected in the public interest. Information provided here may be subject to Freedom of Information requests.

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The NIHR privacy policy includes further information including ways we may use your data, our contact details and details on your individual rights regarding how your data is used. Your data may be shared across the NIHR, including with other coordinating centres, to allow the application to be managed and for statistical analysis, and with external grant reviewers as part of the process for managing the allocation of a grant. This notice is under constant review and will be updated and / or revised based on that review as appropriate.

Table Of Contents

1. Application Summary Information
2. Applicant CV
3. Applicant Research Background
4. Plain English Summary of Research
5. Scientific Abstract
6. Detailed Research Plan
7. Patient & Public Involvement
8. Training & Development and Research Support
9. Detailed Budget
10. Uploads
11. Participants and Signatory
12. Acknowledgement, Review and Submit

1. Application Summary Information

Host organisation (which will administer any award)
Queen Mary University of London

Partner organisation (if applicable)	

Application Type

Strategic Theme	Public health

Partnership Fellowship	N/A

WTE % Please select which WTE option you would prefer
1.00

Duration Doctoral Fellowship is a 3 year award and based on your WTE preference the duration in months is calculated as follows
36

Research Title	The UNITE study: UNderstanding and Improving how blood TEst uncertainty, overuse and overdiagnosis are discussed in primary care – a participatory ethnographic study to improve clinical practice

Research Type	Qualitative study

Proposed start date if grant awarded	01/10/2025

2. Applicant CV

A CV for the Applicant is a mandatory component of this application; please ensure this is completed fully.
Please note that some of the CV questions are automatically completed from information you have provided in the 'Update CV' section of the 'Manage My Details' page. These can be updated via the ARAMIS Home Page or by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

The 'Publication Record' section of the form is automatically populated from publications added into the "My Research Outputs" page of your ARAMIS account.

Degrees and Professional Qualifications

From	To	Qualification	Subject	Country	Institution	Class	Department
01/2021	10/2021	DFSRH	Diploma of the Faculty of Sexual and Reproductive Health	United Kingdom	Faculty of Sexual and Reproductive Health	n/a	-
11/2020	12/2020	DRCOG	Diploma of the Royal College of Obstetrics and Gynaecology	United Kingdom	Royal College of Obstetrics and Gynaecology	n/a	-
09/2020	07/2021	DPMSA	Diploma in the Philosophy of Medicine	United Kingdom	Worshipful Society of Apothecaries of London	Distinction	Faculty of the History and Philosophy of Medicine and Pharmacy
10/2019	08/2020	Mphil	Health, Medicine and Society	United Kingdom	University of Cambridge	First Class with Distinction (76.5%)	Department of History and Philosophy of Science
05/2019	06/2019	AFHEA	Associate Fellowship of the Higher Education Academy	United Kingdom	Higher Education Academy	n/a	-
09/2018	06/2019	Certificate in Learning and Teaching	Certificate in Learning and Teaching	United Kingdom	Queen Mary University of London (QMUL)	Distinction	Queen Mary Academy
09/2015	09/2016	MSc	Reproductive and Sexual Health Research	United Kingdom	London School of Hygiene & Tropical Medicine	Distinction	Faculty of Epidemiology and Population Health
09/2011	06/2017	MBBS	Medicine and Surgery	United Kingdom	The University of Newcastle upon Tyne	Merit (top 10% of graduating class)	School of Medicine

Present and previous positions
(including from/to dates, job title and employer)

From	To	Position	Organisation	WTE % Research
08/2021	-	Academic Clinical Fellow in Primary Care/GP Registrar	Royal Free Hospital NHS Trust	25
07/2020	08/2021	Clinical Fellow in Sexual Health	Homerton University Hospital NHS Trust	0
08/2018	-	Locum Doctor	Barts Health NHS Trust	0

Present and previous positions
(including from/to dates, job title and employer)

From	To	Position	Organisation	WTE % Research
08/2018	08/2019	Honorary Research Fellow	Queen Mary University of London (as Academic Foundation Trainee)	33
08/2017	08/2019	Academic Foundation Trainee	Barts Health NHS Trust	16
08/2015	08/2016	Policy and Advocacy Director	Students for Global Health UK	0
09/2014	09/2016	Co-Chair, National Working Group on Maternal and Child Health	Students for Global Health UK	50

Research Grants held

Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
European University Hospital Alliance Seed-grant Funding	Co-applicant: I co-authored an application for seed-grant funding to facilitate a collaboration (on Type 2 diabetes prevention in women with gestational diabetes) between KCL, Charité University Berlin and Medical University of Vienna.	10000	Charité University/Medical University of Vienna	12	01/12/2022	01/12/2023

Publication Record
(include the list of authors, article title, journal name, and volume)
This information can be edited via the “My Research Outputs” page of your ARAMIS account.

Publications
Elton, L.. (2022) ‘Knowledge, community and care: Digital biocitizenship in gestational diabetes.’, *Sociology of health & illness*, 44(9).
Elton, L.. (2022) ‘Epistemic risk and nonepistemic values in end-of-life care.’, *Journal of evaluation in clinical practice*, 28(5).
Elton, L.. (2020) ‘Non-maleficence and the ethics of consent to cancer screening.’, *Journal of medical ethics*.
Ronaldson, A., Elton, L., Jayakumar, S., Jieman, A., Halvorsrud, K. and Bhui, K.. (2020) ‘Severe mental illness and health service utilisation for nonpsychiatric medical disorders: A systematic review and meta-analysis.’, *PLoS medicine*, 17(9).
Elton, L., Palmer, M. and Macdowall, W.. (2018) ‘Birth order and parental and sibling involvement in sex education. A nationally-representative analysis.’, *Sex Education*, 19(2).
Elton, L.. (2016) ‘Addressing current and future challenges for the NHS: the role of good leadership.’, *Leadership in health services (Bradford, England)*, 29(4).

Relevant Prizes, Awards and other Academic Distinctions
Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
01/08/2021	NIHR	Academic Clinical Fellowship in Primary Care	-

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
01/05/2021	Worshipful Society of Apothecaries of London	Sydney Selwyn Prize, awarded to the best candidate in the Diploma in the Philosophy of Medicine. I was invited to give a lecture on my prizewinning dissertation at a public event.	-
01/10/2020	University of Cambridge	Starred Distinction (80%) in MPhil dissertation, an independent qualitative research project exploring the role and impact of an online support community for women with gestational diabetes.	-
01/10/2018	Royal Society of Medicine Sexuality and Sexual Health Section	2nd prize: trainee and medical student essay prize. Awarded for an essay on the challenges of abortion care provision in the UK.	-
01/09/2018	British Society of Biopsychosocial Obstetrics and Gynaecology	Oral Presentation Prize. Awarded first prize for an oral presentation at the annual conference: "Post-partum depression and intimate partner violence in high-income countries: a review".	-
01/07/2018	North East and Central Thames Foundation School	Merit Award for Teaching Excellence. Awarded for showing the greatest aptitude for teaching in my Foundation Programme cohort.	-
01/08/2017	NIHR	Academic Foundation Programme	-
01/04/2016	Faculty of Medical Leadership and Management	National Student Prize 2016. Awarded first prize in a two-round competition (first round - an essay on contemporary leadership in the NHS; second round - presentation competition).	-
01/06/2015	Beit Trust	Elective Award. Awarded £900 towards the costs of an elective in Harare, Zimbabwe, on the basis of a written application.	-
01/05/2015	Medical Students for Choice	Abortion Training Institute Scholarship. Awarded £1,000 to attend a three-day training institute on abortion care in Phoenix, Arizona, on the basis of a competitive	-

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
		application process.	
01/04/2015	MedAct	"War, violence, peace and health" course scholarship. Given full funding (course fees, travel, accommodation, subsistence) to attend a week-long Masters course at The University of Bergen, Norway.	-

ORCID iD 0000-0003-4621-9131

3. Applicant Research Background

Applicant Professional Background	Medically Qualified

Specialty (if Medically Qualified)
General Practice

Do you have a GMC number?	Yes

GMC number Please supply a valid GMC number	7511242

Please describe your research career to date

I am a qualitative primary care researcher with interests in medical overuse, risk communication, and the role of evidence and values in medical care. I have an academic background (MSc/MPhil) spanning medical sociology, philosophy, and public health. My research is particularly concerned with the social meanings that patients and clinicians ascribe to diagnosis. I have published my research in leading journals, including *Sociology of Health and Illness* and *Journal of Medical Ethics,* and have presented widely nationally and internationally. I have held NIHR academic fellowships throughout clinical training: first as an Academic Foundation doctor and then as an academic GP trainee.

Overdiagnosis research (2019-present)

My MPhil (see below) sparked my interest in how clinicians negotiate risk and uncertainty and how they communicate these issues to patients. I published a single-author paper in *Journal of Medical* Ethics on how to explain the pitfalls of screening to patients (top 5% of all Altmetric-scored outputs). I was invited to present this work at Preventing Overdiagnosis 2022.

I then completed a Diploma in Philosophy of Medicine, exploring in my dissertation how clinicians balance risk when diagnosing patients as dying. This was awarded the Sydney Selwyn prize for the best Diploma dissertation. I was invited to give a public lecture on my research and published it as a single-author paper in *Journal of Evaluation in Clinical Practice* (2022). I presented this work at Preventing Overdiagnosis 2023.

I belong to the Preventing Overdiagnosis Early Career Researchers group and am part of the pre-conference organising committee for the 2025 conference.

NIHR Academic Clinical Fellowship, King's College London (2021-Aug 2025)

I have been a Research Fellow on Dr Andy Guise's UKRI-funded Social Responses to Stigma project since beginning my academic placement in 2023. This ethnographic study examines how stigma and discrimination arise within the health and care services used by people who are homeless in south London. My work includes ethnographic observation in day centres and collaborative analysis of fieldnotes and interviews. I lead a research strand exploring intersections between stigma, homelessness, and health self-management.

Outputs include:

- Presentation at the RCGP Conference 2024.
- First-author paper in preparation (for submission to *Sociology of Health and Illness).*
- Co-author on a quantitative survey of homeless people's stigma experiences, (under review, *BMJ Open)*.
- Co-author of a forthcoming submission on drug use stigma *(International Journal of Drug Policy)*.

In 2022, I collaborated with researchers at Charité University Berlin and Medical University of Vienna on a project to improve Type 2 diabetes prevention in women with gestational diabetes. We secured a £10,000 European

University Hospitals Alliance grant and convened workshops in London, Berlin and Vienna. I am first author of our interdisciplinary commentary paper (provisionally accepted January 2025, *Journal of Midwifery and Women's Health)*.

Other activities:

- PGCert in Applied Research Methods (KCL) (completion June 2025). Awarded Distinction in all completed modules.
- Advisory board member (2022-2024) for PREPARED, an interventional primary care study to improve pregnancy outcomes in women with diabetes.
- Invited peer editor for Professor David Misselbrook's textbook, *Thinking About Medicine: An Introduction to the Philosophy of Medicine*.
- Peer reviewer for *Social Science and Medicine, British Journal of General Practice, Journal of Medical Ethics, BMC Medical Ethics, Digital Health,* and *Information, Communication & Society.*

MPhil, University of Cambridge (2019-2020)

My MPhil (First Class with Distinction) combined medical philosophy, sociology and anthropology. My thesis (Starred Distinction) was a qualitative interview study exploring how women's experience of pregnancy was shaped by participating in an online gestational diabetes support community. I published this work as sole author in *Sociology of Health and Illness* (2022) and presented it at three international conferences, including the International Federation of Gynaecology and Obstetrics World Congress (2021).

Previous research experience

Honorary Research Fellow, Queen Mary University of London (2017-2019):

- Led a systematic review of post-partum depression and intimate partner violence in high-income countries – awarded 1st prize at the British Society for Biopsychosocial Obstetrics and Gynaecology conference (2018).
- Co-authored a systematic review and meta-analysis of health service use by people with severe mental illness, which was published in *PLOS Medicine* (2021).
- Led the quantitative analysis of the LGBT Foundation's survey of women who have sex with women (2019), which was presented at the All-Party Parliamentary Group on Sexual and Reproductive Health.

MSc, London School of Hygiene & Tropical Medicine:

- Quantitative analysis of the National Survey of Sexual Attitudes and Lifestyles, focusing on birth order and sexual health outcomes. This work – published as a first-author paper in *Sex Education* (2018) – attracted significant media attention, including coverage in the Independent and the Daily Mail. (https://www.independent.co.uk/news/education/education-news/sex-education-first-born-children-parents-families-london-school-hygiene-topical-medicine-a8558211.html).

MBBS, Newcastle:

- Co-led a national survey of medical students' views on abortion education, presented at two international conferences (2016). I was subsequently invited to the Royal College of Obstetricians and Gynaecologists to discuss curriculum recommendations.

Teaching and management

In 2018, I co-designed and delivered a critical appraisal module for Global Health BSc students at QMUL. I was also awarded the Foundation School Award for Teaching Excellence in recognition of my mentorship of medical students applying for Academic Foundation posts. I have held Associate Fellowship of the Higher Education Academy since 2019.

From 2015-2016, I was Policy and Advocacy Director for Students for Global Health UK, leading a team of policy analysts and steering four national student research collaborations. As chair of the Cambridge Global Health Lecture Committee (2019-2020) I coordinated a series of six public lectures with over 200 attendees.

Impact of fellowship

A Doctoral Research Fellowship will develop my skills in complex qualitative methodologies and strengthen my

portfolio of research addressing medical overuse. Following a PhD, I aim to secure an Academic Clinical Lectureship to conduct further work exploring the social and relational factors influencing decision-making in primary care, focusing on how to navigate uncertainty and foster shared understanding between patients and clinicians. I am especially interested in co-producing creative outputs to engage wider audiences in considering how medical resources can be used wisely. My eventual goal is to become a Professor of Primary Care, using interdisciplinary qualitative methods to inform excellence in primary care delivery.

Have you already registered for a PhD?	No

Has this application, or a similar application, previously been submitted to this or any other funding body?
Yes

If Yes, provide details
I have submitted an application to the Wellcome-funded NIHR School of Primary Care Research Primary Care Clinicians PhD Programme. I do not yet know whether I have been invited to interview.

Contextual Factors
Please use this section to detail any factors that may have impacted the development of your research career or outputs from your research career to date. NIHR wants to know about any circumstances so that they may take them into consideration during the assessment of your application. Contextual factors may include:

- Career breaks due to parental leave, or periods of illness.
- Reduced time spent undertaking research activities (including research outputs), due to disability or to caring responsibilities. This may include physical or mental health related impairments or conditions, which have impacted your opportunities to undertake research activities
- Reduced opportunities for career support e.g., mentorship, and limited opportunities to undertake prior research and training.
- Impact of the COVID-19 pandemic on your research career

Please also use this section to detail any other factors that may have impacted your research career not listed in the examples provided. The impact on your career to date will be specific to your particular circumstances but could include such impacts as limited opportunities to obtain grant funding or fewer publications. In general terms, contextual factors should be significant, and relevant.

NIHR acknowledges that you may be reluctant, or uncomfortable disclosing relevant information that is sensitive. However, you should bear in mind that we are unable to take into account factors that you do not disclose. Please be assured that the information provided by you is sensitive and will be treated confidentially and in line with the General Data and Protection Regulations (GDPR).

N/A

4. Plain English Summary of Research

Background

Communicating effectively about blood testing is important for ensuring patient safety and involvement in healthcare. Blood tests are useful, but do not always improve patient care. Sometimes they can cause harm: for example, borderline test results may worry patients or lead to unnecessary further tests. This is important because blood tests have become increasingly common: general practice (GP) testing rates have tripled since 2000.

Previous research has shown that patients often feel unsure about why blood tests are done and what results mean. GP staff do not always clearly explain the reasons for testing and the limitations of blood tests, and patients report not having enough opportunity to speak to staff about testing. This means that GP staff and patients may have different expectations and understandings of blood tests. Understanding what happens in consultations about blood tests is important for addressing the rise in testing and ensuring tests are used wisely.

Aims

This project will explore how blood tests are discussed between patients and GP staff, aiming to improve communication about the limitations and risks of blood tests. The goal is for patients to be more involved in decisions about blood testing and understand better what blood tests mean for their health.

Research design
The project has four parts:

1. *Evidence search:* I will search and review existing research on how blood tests are discussed in general practices. This will help to understand what is already known and what is missing.

2. *Observation at GP practices:* I will spend several months watching conversations about blood tests at two different general practices. I will make recordings of patient consultations with GP staff (including GPs, nurses, pharmacists and phlebotomists) to look at how blood tests are discussed.

3. *Workshops with GP staff:* I will show short recordings of blood test discussions to staff. They will consider how blood tests are discussed with patients and think about how communication can be improved.

4. *Theatre workshops with the public:* Professional actors and public contributors will create a short play based on the research. This will be performed to members of the public as part of "Forum Theatre" workshops. Workshop attendees will be invited to make changes to the play to improve communication about blood tests. Public contributors will help to develop the play into two tools to improve blood test communication:
    a. A workshop for general practice staff.
    b. An online education resource for patients.

Patient and Public Involvement (PPI)

People who have recently had blood tests will be involved throughout the project. Nineteen public contributors with diverse ages, ethnic backgrounds, and health experiences helped to shape the project over three workshops. They described many issues with communication about blood tests and felt strongly that this research was important.

A PPI panel will guide the research to make sure the project responds to the needs and preferences of people having blood tests. The panel will meet three times a year to review project plans at each stage and provide input on data analysis. The panel will also write public summaries of the research.

Panel members will be paid for their contributions. They will receive funding for training and attending conferences to share the research. Two lay people will co-chair a separate Project Advisory Group to steer the project.

Sharing the research

I will share the research with local community groups, patients and GP practice staff through social media, blogs, a podcast, and a public event. The online resource for patients will be made available on a project website. I will use

the workshop tool created from the play to train GP staff and trainees on better blood test communication.

5. Scientific Abstract

Background

The use of blood tests in primary care has increased significantly: testing rates have tripled since 2000, and nearly half of all NHS tests are now conducted in this setting. While valuable, blood tests are often over-ordered, leading to high costs (£1.8 billion in 2015-16), increased clinician workload, and potential harm to patients. These harms include anxiety, undue reassurance, and unwarranted further investigations.

Communicating effectively about the limitations, risks, and purpose of blood tests is critical, yet research shows that blood test communication is often inadequate. Clinicians rarely explain the purpose or limitations of tests, while patients often report feeling excluded from testing decisions and unsure about the implications of their results. NHS guidelines for optimising blood testing in primary care focus primarily on test appropriateness, with little attention to how tests are discussed with patients.

Understanding what happens in consultations about blood tests is important for addressing the rise in testing and ensuring that blood tests are used wisely. With patients now able to access blood test results online, these communication issues warrant urgent attention.

Aims

This research will use an ethnographic approach to explore how and whether blood test uncertainty, overuse and overdiagnosis are discussed in primary care settings. It will employ participatory methods to identify how these issues could be better communicated.

Research questions

1. How is communication about blood testing in primary care settings framed in the literature?
2. How, where and when do issues like uncertainty, overuse and overdiagnosis emerge in primary care interactions about blood testing?
3. What challenges do clinicians face in communicating about blood tests, and how can video-reflexive ethnography help clinicians to identify solutions to these challenges?
4. How can participatory performance methods engage patients and clinicians to identify more effective ways of discussing issues with blood tests such as uncertainty, overuse and overdiagnosis?

Methods

A multi-method study across four work packages (WPs):

- WP1: Meta-ethnography synthesising existing qualitative literature on primary care blood test communication.
- WP2: Detailed ethnographic observation in two general practices with contrasting patient populations, focusing on consultations where blood tests are discussed.
- WP3: Video-reflexive ethnography workshops with primary care staff to identify complex challenges around blood testing communication as well as opportunities for change.
- WP4: Forum Theatre workshops with members of the public and primary care staff to develop strategies for improving blood test communication. The findings will inform a co-produced workshop (for clinicians) and an online resource (for patients) to support better blood testing discussions.

Impact

This participatory research project will highlight the primary care consultation as a key site of opportunity for prudent blood testing. It will engage primary care staff and the public in co-producing resources to encourage more effective communication about the limitations of blood tests.

Dissemination

The workshop resource will be shared with primary care staff and GP trainees locally, nationally and internationally. A version of the workshop will be made available as a publicly-accessible resource for patients. Research findings will be published in academic journals, presented at conferences, and shared through blogs and

podcasts.

6. Detailed Research Plan

1. What is the problem?

Effective communication about blood tests is important for shared decision-making and patient safety.[1,2] Primary care blood testing has increased substantially, but patients are often not fully informed about the purpose, risks, or limitations of blood tests. Consultations about blood tests are key moments for involving patients in their care and ensuring the appropriate use of primary care resources. This project aims to improve how primary care staff and patients communicate about blood testing.

Nearly half of all NHS laboratory tests are requested in primary care,[3] with testing rates increasing threefold between 2000-2016.[4] Research suggests that blood tests are often overused: the 2021 *Why Test* study of primary care blood tests found that up to a quarter were unnecessary.[5] Overuse entails avoidable cost and added workload for clinical staff [6,7] and can also lead to overdiagnosis: the identification of abnormalities that would never have caused harm. Borderline results, for example, may prompt anxiety or unwarranted follow-up investigations.[8] Conversely, normal results can provide false reassurance, delaying accurate diagnosis or diverting attention away from patients' presenting symptoms.[9]

Patients need to understand the limitations and risks of blood tests so they can make informed decisions about testing. However, these issues are not always well-explained. GPs recognise that tests are not always useful and may increase rather than reduce clinical uncertainty,[10] but often initiate blood tests without discussing alternatives or explaining the limitations of testing.[11] This means that patients may have different expectations of blood tests. Many patients struggle to understand their results and do not have enough say in decisions about whether to have tests,[12] limiting their opportunities to be involved in their care.

NHS England has produced guidelines for optimising blood testing in primary care but these focus only on the clinical appropriateness of blood tests.[13] They do not address how clinicians should communicate to patients which tests are necessary, nor how to effectively explain the pitfalls of testing. While guidelines are a useful aid for clinical decision-making, they cannot replace the need to explain the rationale behind testing decisions and explore patients' expectations of tests.

Understanding what happens in clinical consultations where blood tests are discussed is crucial to addressing the rise in testing and ensuring that tests are used wisely. Research focusing on these 'on the ground' interactions is missing and is vital to inform improvements in clinical practice. By observing real-world interactions between clinicians and patients, this project aims to identify ways to enhance blood test communication and thereby support more prudent, patient-centred testing practices.

2. Why is this research important?

This research is the first to use ethnographic methods to explore how blood tests are discussed in GP settings. I will directly observe communication between primary care staff and patients, providing important insights into how communication can encourage or hinder appropriate blood test use in the GP setting. I will use innovative participatory methods with primary care staff and the public to identify practical solutions to improve blood test communication.

Primary care staff are generally highly trusted, but some patients do not feel supported to make decisions about their health.[14] Improving how primary care clinicians communicate about the limitations of blood tests is crucial to help patients understand the implications of tests and their results. This is also important for patient safety: for example, ensuring that patients understand that normal test results may not rule out serious illness.

The project responds to concerns about unsustainable workloads in general practice. A survey by the Royal College of General Practitioners (RCGP) found that three-quarters of GPs feel patient safety is being compromised by excessive workloads,[15] while the British Medical Association has warned of 'a growing crisis' in primary care.[16] With the average GP spending an estimated 90-120 minutes per workday reviewing test results,[4] strategies to reduce unnecessary investigations may go some way towards mitigating the clinical and administrative burden of testing.

This research is timely. The NHS Long Term Plan stipulates that all patients must have access to their online records – including test results – by the end of 2024,[17] and online access has been mandated in all English GP practices since 2023.[18] This has significant implications for blood test communication. Online access can create

unintended consequences, including increased workload and patient anxiety about test results.[19,20] This project will offer in-depth insight into how digital access shapes blood test communication.

Wise use of blood testing is important for healthcare sustainability. Primary care clinicians request 45% of NHS blood tests, costing approximately £1.8bn in 2015-16.[4] Unnecessary blood tests can yield avoidable follow-up costs, including hospital visits and investigations.[21] Supporting clinicians and patients to have more candid conversations about the limitations of blood tests may help address medical overuse in the NHS, enabling cost savings and reducing healthcare's carbon footprint.[22]

3. Review of existing evidence

Primary care test requests have increased by 8.7% per year on average since 2000.[4] Testing is expensive[4] and contributes significantly to GP workload.[6] While blood tests can provide important clinical information, many tests requested are of little diagnostic value and do not improve patient care. The *Why Test* study, which reviewed blood test results for 2572 patients across 57 UK GP practices, found that only 6.2% of tests led to a new or confirmed diagnosis, and 48.8% did not result in patient reassurance or any change in clinical management.[5]

Unnecessary and escalating blood testing exposes patients to the risk of overdiagnosis, wherein patients are harmed by further investigations or treatments without receiving any benefit.[7] The potential harms of blood tests may be overlooked compared to more invasive diagnostic procedures, but the sheer volume of blood testing means that these cumulative harms warrant careful consideration. Borderline results are common – in one study of GP blood tests, nearly three-quarters of patients had at least one test result outside the reference range.[5] These results can lead to unnecessary anxiety, particularly if patients access them via online apps.[20] False-positive or borderline results can also lead to 'investigation momentum': further testing or invasive procedures such as biopsies or irradiating scans.[8] Alternatively, normal test results in a patient presenting with symptoms may provide undue reassurance, delay diagnosis, or distract from lifestyle changes.[9]

Despite their limitations, patients have high expectations of blood tests as diagnostic tools[23,24] and may underestimate their potential harms.[25] Patients often request blood tests as a 'check-up' [23] despite limited evidence for the value of many tests in asymptomatic individuals.[26,27] This means that patients and primary care staff may have different ideas about the purpose and value of testing. In a UK-based qualitative study by Watson *et al.*, patients felt that blood tests would provide answers to and solutions for their symptoms; by contrast, GPs were less optimistic about blood tests, viewing them as a way to rule out serious problems rather than pin down specific diagnoses.[12]

Miscommunication between clinicians and patients appears to be an important mediator of medical overuse. Research into antibiotic prescribing practices in primary care, for example, suggests that clinicians often mistake expressions of concern about symptoms as requests for antibiotics.[16] Similar patterns may underlie overuse of blood tests: research shows that clinicians may initiate testing to satisfy or affirm patients regardless of whether there has been an explicit patient request.[17] Where patients do request blood tests, clinicians may acquiesce – even if they feel blood tests are unlikely to be of use – in order to avoid longer discussions about the merits of testing.[9]

Although there is a wide body of research using directly-observed data to examine communication in primary care consultations, [28–33] only one UK-based study focuses on blood test discussions: a secondary content analysis of 27 audio-recordings from an archive of GP consultations. This showed that shared decision-making was rarely evident in blood test discussions.[11] While the study quantified various aspects of poor communication – for example, that patients were given the opportunity to ask questions about blood test results only 13% of the time – nobody has used naturalistic data to explore the nuances of blood test communication in-depth.

Further research is needed to understand how blood test discussions might be improved so that:

1. Clinicians are better equipped to explain to patients why blood tests may (or may not) be necessary.
2. Patients understand what blood tests mean for their ongoing care.
3. Both clinicians and patients can better appreciate and convey the potential benefits and limitations of blood testing.

4. Aims/research questions

Aims

This research will use an ethnographic approach to explore how and whether blood test uncertainty, overuse and overdiagnosis are discussed in primary care settings. It will employ participatory methods to explore how these issues could be better communicated to encourage judicious use of blood testing.

Research questions

1. How is communication about blood testing in primary care settings framed in the literature?
2. How, where and when do issues like uncertainty, overuse and overdiagnosis emerge in primary care interactions about blood testing?
3. What challenges do clinicians face in communicating about blood tests, and how can video-reflexive ethnography help clinicians to identify solutions to these challenges?
4. How can participatory performance methods engage patients and clinicians to identify more effective ways of discussing issues with blood tests such as uncertainty, overuse and overdiagnosis?

The research questions will be addressed in four sequential work packages, each with specific objectives as outlined below.

5. Project plan

Work Package (WP) 1: Synthesising existing qualitative literature on primary care blood testing discussions.

Objectives

1. Identify when, where and how blood tests are discussed in primary care.
2. Explore how patient and clinician perspectives on and experiences of blood testing are framed in the existing literature.
3. Develop new conceptual understandings of blood test communication in primary care settings to inform later work packages.

Methods

I will conduct a meta-ethnography of qualitative literature on communication about blood tests in primary care. Meta-ethnography is a form of interpretive qualitative synthesis involving the re-interpretation of primary data and themes in published research.[34] Unlike other aggregative qualitative synthesis approaches, meta-ethnography aims to develop new theories about the topic under study.[35] This work will therefore build on an existing mixed-methods systematic review of blood test communication.[36] In a preliminary scoping review I identified approximately 20 papers that provide the level of textual detail necessary to support meta-ethnographic synthesis.

I will follow Noblit and Hare's seven-step process for meta-ethnography to generate theoretical interpretations which reflect the qualitative synthesis.[37]

Search strategy

I will systematically search medical, social science and public health databases, using appropriate search terms and operator functions with the support of QMUL library staff. I will also search grey literature via thesis repositories, Google Scholar, citation searches, and reference list reviews. To ensure relevance to contemporary practice, I will limit my research to papers published in English from 2000 to the present, a period which has seen a significant rise in blood testing.[4]

Studies will be imported into Mendeley Reference Manager. After removing duplicates I will screen the titles and abstracts of the remaining papers against SPIDER criteria:[38]

- *Sample:* staff members working in primary care; patients accessing these services.
- *Phenomenon of Interest:* blood testing.
- *Design:* interviews, focus groups, case studies, ethnography.
- *Evaluation:* motivations, thoughts, beliefs, understandings.
- *Research type:* qualitative.

I will screen the full texts of selected papers. A random sample of 50% of these papers will be double-screened by a second reviewer (e.g. Professor Swinglehurst). We will meet to resolve any disagreements.

Data extraction

Study characteristics will be imported into a shared spreadsheet. The second reviewer and I will assess the quality of each study using Joanna Briggs Institute qualitative research criteria.[39] Data will be extracted over multiple readings, capturing themes, first-order constructs (participant quotes) and second-order constructs (the study authors' interpretations).

Data analysis/synthesis

I will work with a second reviewer in the analytical stages of the meta-ethnography and will also consult with qualitative researchers from the QMUL APOLLO team to ensure rigour. Findings will be translated between studies by mapping concepts from one study to another, using a constant comparative method to identify where study findings converge or diverge. Shared themes and concepts from across studies will be integrated into third-order constructs (new interpretations) to develop a 'line of argument'.[34]

I will present my findings using the eMERGe reporting framework developed by France *et al.*[40] These findings will guide my ethnographic approach and analysis in later work packages.

WP2: Understanding blood testing practices and interactions in the primary care setting.

Objectives

To use ethnographic methods to explore in-depth:

1. How decisions to do blood tests are discussed between patients and GP clinical staff.
2. How and whether uncertainty about blood testing decisions or results is communicated.
3. How and whether issues of blood test overuse or overdiagnosis are discussed.
4. The approaches used by GP staff to convey and discuss blood test results.
5. Patients' experiences of using digital tools to access blood test results.
6. The role of language and non-verbal communication in blood test interactions.

Methods

Building on findings from the meta-ethnography, I will conduct an ethnographic study of blood testing practices and communication in the GP setting. Ethnography – involving immersion in a particular setting through observation, interviews,fieldnotes and reflexive diaries – is increasingly recognised as a valuable approach for primary care research.[6,41–44]

Ethnography helps uncover beliefs, behaviours and motivations which may be taken for granted or viewed as unremarkable by patients and clinicians.[45,46] By capturing meaningful details that people might not remember or mention in interview studies,[47] it can reveal the complexity of everyday interactions. This makes it particularly useful for studying safety and quality in healthcare.[46] Ethnographic approaches have been successfully used in research on polypharmacy to develop new understandings of how medical overuse emerges and is sustained through social norms, practices, and interactions.[45,48,49]

Sampling

Participants in PPI workshops felt it was important to explore blood testing practices in diverse patient groups and across different geographical settings. I will therefore purposively select two contrasting GP practices: an inner-city practice with a young, multi-ethnic population, and a suburban practice with an older population. I will use General Practice Fingertips data to identify potential research sites which will be recruited with the support of the NIHR Research Delivery Network.[50]

Data collection

I will undertake observation in each site 2-3 times per week over 4-6 months (approximately 200 hours in total). This schedule, informed by similar studies,[6,43,48] balances the need for deep immersion in the field with the practical constraints of being a lone researcher.

Data collection will include:

1. *Observation of professional practices*

- I will observe clinical consultations, reception/administrative areas, interprofessional meetings, and Patient Participant Groups. Only half of primary care blood tests are ordered by GPs,[5] so I will also observe pharmacists, nurses and phlebotomists to ensure the dataset reflects the diversity of primary care blood testing interactions. I will make detailed fieldnotes during and after observations.

2. *Ethnographic interviews with patients and GP staff*
   - I will conduct informal ad-hoc interviews with patients and GP staff, which will be audio-recorded (with consent) or included in fieldnotes.
   - To explore digital communication about blood tests, I will conduct 'think-aloud' interviews, inviting patients and staff to describe their thoughts as they use digital resources like the NHS app.[6,51]

3. *Gathering written communication relating to blood testing*
   - I will gather and analyse relevant written materials, e.g. consultation notes, screen captures from patient apps.

4. *Recording consultations*
   - I will record approximately 45 consultations/'think-aloud' interviews covering key stages of the blood testing process, e.g.:
     a. discussing/ordering tests;
     b. receiving/filing blood test results;
     c. communicating results to patients.

Dataset diversity

I will ensure that a range of primary care clinicians are represented in the recordings, and will include consultations with patients of different ages, sexes and ethnicities, as well as those with multimorbidity and mental illness.

I will also record consultations involving interpreters and patient advocates/carers. I have made provision in the budget for patient information and consent forms to be translated into the three most common patient languages (besides English) in each research site.

Consent

I will seek consent for recording from patients and clinicians either prospectively (for face-to-face consultations) or retrospectively (e.g. if seeking to access audio consultation data from practice telephone systems). Participants in PPI workshops were surprised that many GP practices automatically recorded telephone consultations and felt patients needed to be told this before being approached for consent. I will consider this in the consent procedure, with input from the PPI panel and research sites. Clinicians and patients will be able to withdraw their consent for recording during the consultation or request that the recorded data be destroyed at any point afterwards.

I discuss the feasibility of recording further in Section 9.

WP3: Exploring and addressing professional concerns around blood test communication.

Objectives

1. Elicit the perspectives of primary care staff on the challenges of communicating about blood tests, especially when dealing with uncertainty or potential overdiagnosis.
2. Provide a safe and supportive environment for clinicians to trial and refine alternative communication strategies.

Methods

Video-reflexive ethnography (VRE) is an innovative participatory research method which involves filming people *in situ* doing their usual work, then showing the footage back to them in facilitated workshops.[52] Participants in VRE workshops watching these recordings see familiar practices from a fresh perspective, stimulating discussions about what they may have taken for granted and where they might see possibilities for change. This approach means that those delivering care are actively involved in the process of data analysis and quality improvement.

VRE has been successfully used in various healthcare settings, including maternity care, [53], acute medical units,[54] and dementia services.[55] My supervisors (Professor Swinglehurst/Dr Fudge) have also used VRE to explore primary care conversations about polypharmacy – another aspect of medical overuse.[56]

Workshops will be conducted according to the four key principles of VRE:[52]

1. *Exnovation:* acknowledging the complexity of existing practices and encouraging innovation with existing resources.
2. *Collaboration:* valuing participants' expertise and supporting a process of co-creation.
3. *Reflexivity:* encouraging participants to re-evaluate their assumptions, beliefs and practices.
4. *Care:* creating a supportive environment for participants and researchers.

Data collection

I will run 6 one-hour workshops (3 per site), with 6-10 primary care staff in each. I will schedule these in practice training time to maximise engagement. Workshops will include a mix of staff including GPs, nurses, pharmacists, phlebotomists, and practice managers.

I will show selected short recordings from WP2 to prompt interprofessional discussions about blood testing conversations in primary care. The chosen clips will focus on 'critical moments' in blood test consultations which I will select based on iterative analysis and the findings of the meta-ethnography.

The workshops will provide an opportunity for primary care staff to identify good practices and rehearse a range of strategies to support improved communication with patients. Workshop discussions will be audio-recorded and transcribed for analysis.

WP4: Using participatory performance methods to explore how to improve blood test communication

Objectives

1. Co-create an interpretation of study findings with the PPI panel and professional actors.
2. Bring together members of the public and primary care staff to develop strategies to improve blood test communication.
3. Co-produce resources, including a workshop and public engagement materials, to help improve communication about the limitations of blood tests.

Methods

Forum Theatre is a participatory performance method allowing participants to enact, critique and transform real-world scenarios. It is one of the tools for social change described in Augusto Boal's *Theatre of the Oppressed*, developed in the 1960s/1970s with marginalised communities in Brazil.[57]

In a Forum Theatre workshop, actors perform a scene involving conflict or misunderstanding while audience members ("spect-actors") observe the social dynamics which emerge.[58] A facilitator then invites spect-actors to suggest or enact alternative actions to change the scene, improving the dramatised interaction through iterative improvisation.[58]

Forum Theatre is gaining traction in research exploring how interpersonal interactions can be improved in healthcare.[59–61] It is particularly effective for highlighting how power dynamics and inequalities can impact upon communication. Like VRE, it provides a space for participants to see situations from alternative perspectives and develop new practices collaboratively.

Co-production process

Part 1

A day-long workshop with the PPI panel and actors from Improbable, an improvisation theatre company. Using selected research materials (e.g. ethnographic fieldnotes, VRE workshop excerpts), workshop participants will co-design a short play about blood test discussions.

Part 2

Two Forum Theatre workshops, each with approximately 20 participants, including the public and primary care staff. Public participants will be recruited in collaboration with Social Action for Health, a charity working closely with East London communities affected by health inequalities. Led by Improbable, these workshops will explore how to improve blood test discussions through audience suggestions and interventions.

Part 3

Another day-long workshop with the PPI panel and Improbable actors. A videographer will guide the recording of short scenes based on the play and Forum Theatre workshops. The PPI panel will help develop these video clips into two resources to disseminate study findings and support improved blood test communication: a workshop for primary care clinicians and an online resource for patients (see Section 6).

Analysis

This study will generate a rich dataset, including fieldnotes, audio/video-recorded consultations, VRE workshop transcripts, and reflexive diaries. I will take an interpretive approach to analysis, combining insights from ethnographic observation with close study of how language is used *in situ* by patients and GP staff.[62] This linguistic ethnography orientation offers distinctive insights into the interpersonal dynamics involved in working practices and how these shape and are shaped by institutional norms. Widely used in education research, linguistic ethnography is increasingly recognised as a valuable methodology for understanding the role of language in medical interactions, including in primary care. [41,49,63]

My analysis will be grounded in relevant social theory, underpinned by an understanding of diagnosis as a social phenomenon as well as a clinical tool.[64] I will draw upon work by scholars of diagnosis such as Annemarie Jutel[64,65] and Charles Rosenberg,[66] as well as Deborah Lupton's work on the sociology of risk.[67]

Data will be managed in NVivo 15. I will use CLAN software to transcribe recorded consultations, using Conversation Analysis conventions to capture fine-grained interactional details such as pauses and overlaps.[68]

6. Dissemination, outputs and anticipated impact

Clinician/patient resources based on Forum Theatre workshops

Workshop for clinicians

- The workshop will use the recorded scenes from WP4 as prompts for attendees to consider how blood tests are discussed and how this could be improved.
- I will deliver the workshop in teaching sessions for local GP practices and VTS schemes, seeking early feedback to refine the workshops and demonstrate impact.
- I will run the workshop at GP conferences to ensure the resource reaches clinicians nationally and internationally.
- I will share the workshop through academic GP networks (see Collaborations) and with GP VTS Programme Directors.

Online resource for patients

- Hosted on the project webpage, this online resource will feature the recorded clips of blood test communication developed in WP4. It will include written guidance to help patients think about the purpose and possible outcomes of blood tests and consider the questions they want to ask primary care clinicians.
- The resource will be promoted via social media (e.g. Bluesky/LinkedIn), flyers in community groups/GP practices, and Patient Participant Group mailing lists.
- Key messages will be also be disseminated to patients via leaflets, newsletters, and GP waiting room displays.

I will apply for postdoctoral funding to further develop and evaluate these resources.

Dissemination of study findings

To patients/the public:

Study findings will be shared with the public via:

- A project webpage, featuring blogs and the PPI panel's summaries of the research plan and findings
- Project news updates via Bluesky and LinkedIn.
- An episode of the QMUL Wolfson Institute podcast, *Thinking In Between.*
- A stall at QMUL's Festival of Communities, an annual public engagement event which has showcased QMUL research to over 25,000 attendees since 2016.

To academics/clinicians:

- Publication of at least four peer-reviewed open-access papers in target journals:
  - Meta- ethnography *(Sociology of Health and Illness)*
  - Linguistic ethnography *(Social Science and Medicine)*
  - Video-reflexive ethnography *(BMJ Quality and Safety)*
  - Forum Theatre *(BMJ Evidence-Based Medicine)*
- National conference presentations, e.g. Society for Academic Primary Care, Royal College of General Practitioners, Health Services Research UK.
- International conference presentations, e.g. Preventing Overdiagnosis (a multidisciplinary conference well-attended by GPs), WONCA (World Organization of Family Doctors).
- Presentations in clinical/academic networks, e.g. RCGP Optimal Testing, University of Bristol Centre for Academic Primary Care.

To all stakeholders:

- Supported by the QMUL Centre for Public Engagement, I will host a public dissemination event for local clinicians, community groups and the public to share the findings and discuss how best to implement the WP4 resources.

Anticipated impact

This research will:

1. Encourage discussions between patients and clinicians about the challenges of blood testing and how they can be overcome.
2. Foreground primary care consultations as key opportunities for the safe and prudent use of blood testing.
3. Develop a 'good practice' approach to discussing blood tests in primary care, co-created with patients.

Key impacts:

For patients: They will better understand the limits of blood tests, feel more confident in making shared decisions about testing, and have a clearer grasp of the implications of their results.

For primary care staff: They will gain confidence in explaining the risks and limitations of blood tests, improving their ability to discuss testing decisions and results with patients.

For the NHS: Better communication and shared decision-making could improve patient safety and reduce unnecessary testing. This will reduce blood testing costs, ease administrative workloads, and reduce the costs and carbon footprint associated with repeat/follow-up investigations.

7. Project management

I will oversee the project's progress, budget and outputs with support from my supervisors and the QMUL finance team. I have uploaded a detailed research timetable.

Progress will be monitored through:

Supervision

- Fortnightly meetings with Professor Swinglehurst.
- Monthly meetings with Dr Fudge.
- Monthly meetings with Dr Watson (online, with in-person meetings in Bristol twice-yearly).
- Full supervisory team meetings three times a year, with a yearly face-to-face meeting in London.
- Email support and feedback on written work from all supervisors as required.

Project Advisory Group (PAG)

The PAG will meet online twice a year to assess and guide study progress. PAG members will also provide mentorship on an ad-hoc basis. The PAG will be co-chaired by lay members Steven Oliver and Cathy Mullins, who attended pre-proposal PPI workshops. Both have significant public involvement experience, including chairing meetings, and will receive funding for relevant training.

The other PAG members are:

- Professor Natalie Armstrong (Executive Dean of the School of Health and Medical Sciences, City St George's)
    - Medical sociologist with expertise in applied qualitative research and overdiagnosis.
- Dr Julian Treadwell (GP & NIHR Academic Clinical Lecturer, University of Bristol)
    - Lead of GP Evidence, a co-produced information resource for GPs on treatment benefits/harms.
- Dr Margaret McCartney (GP & Senior Clinical Lecturer in General Practice, University of St Andrews)
    - Internationally-recognised advocate for evidence-based medicine, with extensive broadcasting experience.
- Professor Stephen John (Hatton Lecturer in the Philosophy of Public Health, University of Cambridge)
    - Medical philosopher with interests in risk communication and early cancer detection.
- Dr Jessica Drinkwater (GP & NIHR Academic Clinical Lecturer, University of Manchester)
    - Academic GP with experience of participatory and applied theatre methods.

Formal reviews

As per QMUL regulations, I will have yearly reviews to assess progress and identify additional support/training needs.

8. Ethics

I will obtain ethical approval from QMUL and the NHS Health Research Authority before starting any data collection involving participants. Key ethical considerations include:

Ethnographic observation and recording

I will get written consent from all GP practice staff for observation. My presence will be respectful and unobtrusive and I will leave a space immediately if asked. GP staff will seek a patient's explicit consent for me to observe their care, ideally without me present (e.g. when collecting patients from the waiting room).[69] I will clearly explain my dual role as a researcher and practising GP.

Video-reflexive ethnography workshops

Staff may feel uncomfortable watching or hearing comments about their consultations. I will mitigate this by ensuring that staff are aware of the purpose of the workshops: i.e., to collectively identify new ways of working, rather than critiquing each other's practice. I will work with Professor Swinglehurst and Dr Fudge to plan these sessions.

Confidentiality

This project involves video/audio-recordings of clinical consultations. Ensuring the confidentiality and security of data is thus essential; this was also a high priority for PPI participants. All research data will be securely stored in line with QMUL's data management policy, the Data Protection Act 2018, and Research Ethics Committee requirements. Recordings will be encrypted, securely stored, and deleted from recording devices after transfer.

9. Success criteria

I will measure the success of my PhD according to the completion of the milestones described in the attached Gantt chart. Key indicators include:

1. Ethical approval secured by end of year 1;

2. Meta-ethnographic synthesis submitted for publication by 1st quarter of year 2;
3. Ethnographic data collection/VRE workshops complete by 3rd quarter of year 2;
4. Co-produced resources for patients/clinicians finalised by 3rd quarter of year 3;

Risks and mitigation

1. Delays in obtaining ethical approval: I will apply for ethical approval promptly upon starting the fellowship. I will be supported by my supervisors to prepare a high-quality application to reduce the chance of rejection.

2. Difficulties completing the meta-ethnography: I have done a rapid scoping search which confirms there is a suitable body of relevant literature. I will attend two training courses and will work with a QMUL librarian to prepare the search strategy. Members of the APOLLO team with experience in meta-ethnography will be able to offer methodological support.

3. Difficulty gaining consent to record consultations: Research shows high rates of patient consent to video-recording of GP consultations for research purposes.[70] Participants in the pre-proposal PPI workshops also felt that recording consultations would be acceptable. I will seek the PPI panel's guidance on recruitment materials including consent forms. I will draw upon my supervisors' experience of obtaining consent from clinicians to record consultations[56] and will use the period of ethnographic observation to build trust with GP staff.

4. Challenges in the applied theatre workshops: Forum Theatre may be unfamiliar to participants and they might feel unsure about the aims or process of the workshops. This risk will be mitigated by the Improbable actors' expertise in improvised participatory theatre. I will be supported by my mentor, Dr Drinkwater, who has experience of using applied theatre methods.

7. Patient & Public Involvement

Please describe how patients/service users, carers and the public have been involved in developing this proposal

To develop this proposal, I held three workshops with 19 members of the public who had recently had general practice blood tests. Participants were from diverse ethnic backgrounds, including Black British, South Asian and Chinese, and ranged in age from 20s-70s. Several had long-term conditions and/or identified as disabled, and two participants were neurodivergent.

In the first workshop I asked about participants' experiences of blood testing, which helped shape my research questions. In later workshops, we discussed the proposed research: the topic, aims and objectives, methods, and plans for sharing findings. Participants felt strongly that this research was important. They highlighted communication difficulties throughout the blood testing process:

- *"I don't feel the GP discusses the decision to do a blood test… they just say you need one."*
- *"There's a lot of difficulty in understanding the results without having to go onto Google."*

They also described concerns about blood test uncertainty or overuse:

- *"It's confusing getting a borderline result - the doctor says we don't worry when the test is just out of range, but there's no time for them to properly explain."*
- *"The GP says you haven't had that blood test in a while and they order it whether you need it or not."*

Participants liked the idea of using applied theatre methods and felt that a play would be an accessible way of sharing the research. Based on feedback from these workshops, I made the following changes to the project:

- I will observe Patient Participation Groups (PPGs) as part of the ethnography and share my findings in PPG meetings to involve existing patient networks in the research.
- Participants wanted the workshop resource for GPs to be part of GP training, so I will run the workshop for local GP training programmes.
- I decided not to include GPs in the workshop session to develop the play, as participants felt that PPI contributors would be able to participate more freely without staff present.

Two workshop participants co-wrote the plain English summary and provided feedback on the PPI plans. They will co-chair the Project Advisory Group.

Please describe the ways in which patients/service users, carers and the public will be actively involved in the proposed research, including any training and support provided

I will assemble a Patient and Public Involvement (PPI) panel to guide the project, made up of people who have had a blood test in primary care in the past year. I will ensure panel members are diverse in age, ethnicity and gender and have a range of health experiences.

I will meet the panel three times a year to discuss study progress and troubleshoot any issues which come up. All panel members will be paid for their time and travel expenses in line with NIHR guidance. I will also offer payments for childcare/caring expenses. The research budget includes funding for panel members to access training and present the research at conferences.

I will seek the panel's guidance on recruiting research sites and obtaining consent for recordings. I will also gather their feedback on early findings from observations. This will help me choose recordings to show in workshops with primary care staff.

The PPI panel will work with a theatre company to develop a play based on the research findings. This will used in theatre workshops with members of the public to find ways to improve blood test communication. I will recruit participants for the workshops in collaboration with Social Action for Health, a local charity with extensive experience of working with East London communities who are not well-represented in research.

Together with the panel and theatre company, I will create short video clips based on the workshops. The panel will help me to adapt these into a training workshop (for clinical staff) and a public-facing online resource (for patients), both aimed at improving communication about blood tests. The panel will also develop summaries of the research plan and findings to post on a project webpage. At the end of the project, the PPI panel will help to host a

public event to share our findings with members of the community and local primary care staff.

Two public contributors helped to develop this proposal and will co-chair a Project Advisory Group which includes my supervisors and four external academics/GPs. This group will provide scrutiny, guidance and oversight throughout the project.

If it is considered not appropriate and meaningful to actively involve patients/service users, carers and the public in your proposed research, please justify why
n/a

8. Training & Development and Research Support

Proposed training and development programme

The QMUL Doctoral College and the Wolfson Institute of Population Health offer doctoral development programmes with courses covering project management, personal effectiveness, and research governance. I will attend induction events, yearly cohort days, and research skills seminars. As outlined below and in the attached training timetable, I have planned further training to allow me to successfully complete the PhD and continue my development as a clinical researcher.

Research skills

Qualitative methodology:

- Advanced Qualitative Research Methods (8 weeks part-time, University of Oxford, £3175): Masters module focusing on more complex aspects of qualitative research such as reflexivity, bias, and advanced data collection methods. This will underpin the qualitative methods training I received in my MSc and MPhil.

Meta-ethnography (WP1):

To refresh my skills in systematic review and develop my understanding of qualitative synthesis, I will take the following courses:

- Literature Searching for Systematic Reviews (half-day, QMUL): course on literature searching in preparation for the meta-ethnography database searches.
- Overview of Qualitative Review and Synthesis Methods (1 day, University of Bristol): course to aid me in conducting an interpretive qualitative synthesis.

Ethnography and video-reflexive ethnography (WPs2&3):

Building on my previous experience of ethnographic research, I will take the following external courses:

- Ethnography and Ethnographic Methods (5 days, Essex Summer School in Social Science Data Analysis, £1,600): module to provide a thorough introduction to the theory and practice of ethnographic research and set me up for ethnographic observation in GP practices.
- Summer school module on Linguistic Ethnography (5 days, Ghent University, £250): an introduction to key concepts in linguistic ethnography. This will support me in the close analysis of recorded data in WP2, building on the Conversation Analysis course (University of Oxford) I completed in 2024.

Creative co-production (WP4):

I will be mentored by Dr Jessica Drinkwater, a member of the Project Advisory Group who has experience of using forum theatre to improve primary care services. I will also take the following courses:

- Creative Methods in Qualitative Research (2 days, Social Research Association, £220): course exploring creative methods of data collection and analysis.
- Forum Theatre Intensive Training (5 days, School of Theatre of the Oppressed, £850): intensive course to develop skills in applied theatre methods.

Dissemination and impact

Patient and public involvement/engagement:

To support me in working with public contributors throughout the project I will take the following courses:

- Co-design, Co-production and Participatory Action Research; Inclusivity within Research Processes (1 day each, University of Bristol)
- Hands on Engagement: Taking Research to Events and Festivals (1 day, QMUL): course to prepare me to host the dissemination event.

Implementation of findings:

- Knowledge into Action (8 weeks part-time, University of Oxford, £3175), led by Professor Carl Heneghan. This module will support my plans for impact and dissemination, helping me to understand how my recommendations can be implemented in clinical practice. It will also inform an application for postdoctoral funding to develop and evaluate interventions to improve blood test communication.

Disseminating findings:

- Science Communication Masterclass (4 days, UWE Bristol, £400) : intensive course on communicating research findings with the public. This will help me in sharing my PhD research via podcasts, blogs and social media.
- WordPress website design (1 day, CityLit, £129): course to help me design a project website to share study findings and patient resources.

Conferences:

I will attend and present work at national/international conferences including:

- Royal College of General Practitioners
- Society of Academic Primary Care
- World Organization of Family Doctors (WONCA)
- Preventing Overdiagnosis: an international conference with a strong primary care presence

Development as a researcher

Leadership:

- Emerging Clinical Leaders (6 days in total, King's Fund, £2712): interprofessional programme to build skills in leadership, resilience and team management. This will further my development as an independent research leader.

Teaching:

- To develop as an educator I will facilitate undergraduate/Masters module seminars within the department when my research commitments allow. I will attend Graduate Teaching Assistant Development workshops provided by QMUL.

Research networks:

I will be a member of the following research networks, allowing me to learn from other researchers, get feedback on my work, and develop new collaborations:

At QMUL:

- APOLLO Social Sciences team:
  - Multidisciplinary team of qualitative primary care researchers, led by Professor Swinglehurst. Alongside regular team meetings I will have the opportunity to attend APOLLO's termly writing retreat.
- Thinking Between the Lines:
  - Qualitative research network, co-convened by Dr Fudge, holding monthly meetings for researchers to present work-in-progress and receive feedback.
- Curiosity Workshops:
  - Monthly gatherings to share and seek advice on qualitative research in-progress, led by Professor Swinglehurst.

Externally:

- Preventing Overdiagnosis Early/Mid-Career Researcher Network:
  - International network of researchers working on overdiagnosis, which provided helpful feedback on this proposal.
- RCGP Optimal Testing group:
  - Subgroup of the RCGP Overdiagnosis interest group, led by Dr Watson. I will attend their tri-yearly meetings to share my work and discuss how to improve primary care blood testing practices.

Research visit

In the 3rd year of the PhD, I have been invited to spend six weeks in Australia visiting two research departments. Both departments are pioneering work on the communication of overdiagnosis to patients. In each site I will be mentored by the unit lead and a mid-career researcher. We will agree a timetable in advance of my visit.

- Sydney Health Literacy Lab, University of Sydney
  - The Lab focuses on healthcare communication to prevent overdiagnosis and low-value care. I will spend four weeks here, hosted by Professor Carissa Bonner, a behavioural scientist who specialises in health literacy and risk communication, and Dr Brooke Nickel.
  - I will use insights from the Lab's extensive work co-producing health literacy tools to aid the development of the WP4 resource for patients.
- Australian Centre for Health Engagement, Evidence and Values (ACHEEV), University of Wollongong.
  - I will spend two weeks at ACHEEV, hosted by Professor Stacy Carter and Dr Patti Shih. The ACHEEV team aims to find solutions to medical overuse by making evidence more accessible.
  - I am a technical advisor for Dr Shih's project developing an online resource to explain blood test results to patients, and will be able to develop this collaboration during my visit.

RESEARCH SUPPORT / SUPERVISOR 1

Name	Professor Deborah Swinglehurst

Institution	Queen Mary, University of London

Position	Professor of Primary Care

Description of support provided
Deborah Swinglehurst is a Professor of Primary Care and general practitioner. Her academic work uses qualitative methods, including ethnography and linguistic ethnography, to explore how interaction and social relationships shape healthcare practices. She leads the APOLLO social science group, adopting innovative methodologies at the interfaces of medicine, social science and linguistics to address complex healthcare challenges. She is Institute co-lead for Applied Social Science and sits on the Executive Board and Research Committee. She is in the senior management team for the Wellcome-funded Doctoral Training Programme on Health Data in Practice and the leadership team for QMUL's NIHR SPCR membership. Professor Swinglehurst has published widely in journals including *BMJ, BMJ Quality & Safety, BJGP*, and *Social Science and Medicine*. She is on the 2024 Stanford/Elsevier Top 2% Scientists list of the most cited scientists globally and was awarded the 2019 RCGP/SAPC John Fry Award for her contribution to general practice. Her recent NIHR-funded research on multimorbidity and polypharmacy involved co-producing a suite of RCGP e-learning modules (reaching >1000 learners) and a public engagement resource (*Let's Talk Differently about Medicines*) which has reached >1800 people. She will provide expertise in meta-ethnography, ethnographic approaches (including linguistic ethnography/video-reflexive ethnography), and creative co-production. We will have supervision meetings fortnightly and she will provide further support as needed. Accredited by the UK Council for Graduate Education as a recognised research supervisor, she has supervised 6 PhD students to completion, currently supervises 6 PhD students, and has examined 10 PhDs.

RESEARCH SUPPORT / SUPERVISOR 2

Name	Dr Jessica Watson

Institution	University of Bristol

Position	NIHR Academic Clinical Lecturer in General Practice

Description of support provided
Jessica Watson is a general practitioner and NIHR Academic Clinical Lecturer whose research uses mixed methods to understand and improve primary care blood testing practices. She chairs the Primary Care Academic Collaborative (PACT) and leads the Optimal Testing division of the RCGP Overdiagnosis group. Dr Watson has been awarded three consecutive NIHR Fellowships and has delivered over £2m of research funding. She has authored 64 peer-reviewed research articles which have been cited over 2,100 times. Her work has been featured on BBC radio, ITV News, and in *BMJ, BJGP* and *RCGP* podcasts. She was Principal Investigator on the NIHR-funded Test Talk Tools study (2022-2024), and currently leads two further NIHR-funded studies: • Accessible Results (Principal Investigator): qualitative project developing tools to help patients with diverse needs to access and understand online blood test results. • Optimal testing for long-term condition monitoring in primary care (Joint Lead Applicant): mixed-methods study to develop, implement and evaluate a new blood testing strategy for long-term conditions. Dr Watson will provide expertise in blood testing and overdiagnosis as well as mentorship in research dissemination. She will support me to collaborate with researchers at the University of Bristol and within PACT. We will meet online monthly, with twice-yearly meetings in-person in Bristol. We will have joint online meetings with Professor Swinglehurst and Dr Fudge three times a year to ensure regular contact between all supervisors. She is supervising 1 PhD student and has examined 3 PhDs. She has supervised 2 GP ACFs and 3 MSc students.

RESEARCH SUPPORT / SUPERVISOR 3

Name	Dr Nina Fudge

Institution	Queen Mary, University of London

Position	Lecturer in Social Science

Description of support provided
Dr Fudge is a Lecturer in Social Science and THIS Institute Research Fellow with a background in social anthropology. Her research explores knowledge, expertise and practice in complex healthcare settings such as community pharmacies and GP practices. Her recent research focuses on patients' and clinicians' experiences and practices of polypharmacy, aiming to develop safe and person-centred approaches to primary care medicines use. She has extensive experience of qualitative participant-focused methods, including narrative, ethnographic and visual approaches. She has particular expertise in video-reflexive ethnography (VRE) which she has used in multiple polypharmacy research projects, including her THIS Fellowship (awarded 2021), which uses VRE to improve the delivery of structured medication reviews. She has worked closely with Professor Swinglehurst since 2017 as part of the APOLLO-MM project, including on the co-production of patient-facing resources to encourage discussions around polypharmacy. She is Deputy Director of Graduate Studies for the Wolfson Institute of Population Health and co-convenes Thinking Between the Lines, a qualitative research network through which I will receive feedback on my work and take part in data analysis workshops. I will meet with Dr Fudge on a monthly basis throughout the fellowship. She will provide particular guidance on hosting the VRE workshops in WP3 and analysing the resulting data. With her experience of co-design methods she will also support me in co-producing the clinician and patient resources in WP4. She is currently supervising 3 PhD students (1 with Professor Swinglehurst) and has supervised 4 PhD students to

completion (2 with Professor Swinglehurst).

Collaborations

Academic

Centre for Academic Primary Care (CAPC), University of Bristol

Through Dr Watson, I will develop links with CAPC and their ongoing blood testing research, including the Accessible Results study – a participatory study exploring how people with diverse needs can be supported to access blood test results online. Collaborating with CAPC will enable me to foster a cross-institutional partnership on a project Professor Swinglehurst is developing which explores GP blood testing organisational routines.

Primary care Academic Collaborative (PACT)

PACT – a network of UK primary care researchers – has run several studies relevant to my proposed PhD: the *Why Test* study, investigating the rationale behind primary care blood tests; and the *Hidden Workload* study, exploring clinician workload. I will collaborate with PACT to disseminate the research findings, including the workshop for GP staff, and will explore the possibility of a national spin-off project exploring blood test communication.

Sydney Health Literacy Lab/ Australian Centre for Health Engagement, Evidence and Values

These two centres – both part of Wiser Healthcare (https://www.wiserhealthcare.org.au/), a global initiative to reduce overdiagnosis – will provide a rich environment for collaboration. My research visit will facilitate a mutual exchange of methodological approaches. At the Sydney Health Literacy Lab, I will learn from behavioural scientists about implementation science and patient resource development. At ACHEEV, I will explore their empirical ethics work, particularly their use of community juries to evaluate overdiagnosis. In return, I will share my experience of using ethnographic methods to understand how overdiagnosis is discussed in clinical practice.

Clinical

Dr Stefán Hjörleifsson, Choosing Wisely Norway/University of Bergen

Dr Hjörleifsson, a Norwegian GP and academic, leads Choosing Wisely Norway, an initiative promoting discussions between clinicians and patients about the necessity and value of investigations and treatments. There is strong professional commitment to reducing unnecessary testing in Norway, and in 2016 the Norwegian College of General Practice was the first national medical college worldwide to adopt a position on overdiagnosis. I am collaborating with Dr Hjörleifsson on workshops for WONCA 2025 and the Nordic Congress of General Practice 2026. He has invited me to spend two weeks visiting him at the University of Bergen in the 2nd year of my PhD. I will engage with their GP academic community, learn about their work advocating for prudent testing in Norway, and explore how these strategies could be adapted to a UK context.

Lab Tests Online (UK), NHS Digital, and Pathology Tests Explained (Australia)

My supervisor Dr Watson works closely with Lab Tests Online, while my ACHEEV mentor, Dr Shih, is undertaking a project with Pathology Tests Explained (the Australian counterpart), on which I am a technical advisor. These organisations provide online resources to help patients understand their blood test results. Dr Watson is also collaborating with NHS Digital on the Accessible Results study, to provide guidance on NHS app design for online results access. I will work with these three organisations to integrate my research findings and offer recommendations of how online blood test resources could be improved.

Community

Social Action for Health (SAfH): East London community health charity.

SAfH have extensive experience running community health research projects and focus groups with NHS and academic partners. I will work with SAfH to assemble participants for the Forum Theatre workshops in WP4.

Improbable: Improvisation-based theatre company (https://www.improbable.co.uk/)

Known for its award-winning productions, Improbable also operates a research wing that uses improvisation to interpret research findings and bring them to new audiences. Ellie Claughton, Improbable's Executive Director, has agreed to collaborate on the project. Improbable's theatre-makers will help to design the play and facilitate the Forum Theatre workshops.

Host Organisation Support Statement
The following statement of support must be completed by the head of your lead academic department. In order to contribute to this application, this individual must be invited to do so via the 'Participants and Signatories' section of the form.

Statement of support

As Deputy Director of Wolfson Institute of Population Health (WIPH), I am delighted to support Lotte's application for an NIHR Doctoral Fellowship.

Lotte has already made significant contributions to academic primary care with her ethnographic study of stigma experienced by homeless people during her GP Academic Clinical Fellowship at King's College London. She has won numerous prizes, has a strong publication record, and has already accrued considerable expertise in leadership, teaching and management. She clearly has aptitude for ethnographic methodologies, interdisciplinary scholarship, and a passion for primary care. A Fellowship will enable her to grow as an independent researcher. Her proposal extends research being conducted in both our Centre for Primary Care and our Centre for Cancer Screening, Prevention and Early Diagnosis and aligns with our commitment to our cross-cutting theme (co-led by Professor Deborah Swinglehurst) on Applied Social Science in Health. Lotte has developed her application over many months, meeting regularly with Prof. Swinglehurst and Dr Nina Fudge, and has twice joined their group on writing retreats to support this development.

The Centre for Primary Care (CPC), where Lotte will be based, is a vibrant, multidisciplinary group with interests spanning multimorbidity, complex interventions, data science, health inequalities, sustainable environments, healthcare interactions, and complex systems. The CPC specialises in adopting methodologies - quantitative and qualitative - that address the complexities of generalist clinical practice.

The CPC leads the university's membership of the prestigious NIHR School for Primary Care Research, which focuses on facilitating high quality research through strategic leadership and ringfenced funding, and on building capacity through an established training programme. Lotte will have access to the training resources, networks and opportunities across the nine HEI members of the SPCR, with strong links to the NIHR Schools for Social Care Research and Public Health Research. We will also support Lotte in developing a post-doctoral programme of work. Prof. Swinglehurst has supported recently completed PhD students with successful post-doctoral Fellowships, seed/bridging awards, mentorship, honorary academic titles and grant-writing retreats.

SPCR membership offers numerous opportunities for collaboration across HEIs. This proposal marks an exciting new collaboration with Lotte's secondary supervisor, Dr Jessica Watson (Bristol), an academic GP who has published widely on investigations in primary care. It has already sparked the possibility of further QMUL-Bristol collaborations to research overdiagnosis and medical overuse – an issue of pressing concern in the quest for sustainable primary care.

Lotte's training will be overseen by an experienced supervisory team. Prof. Swinglehurst has an international reputation for qualitative research in primary care, with special expertise in ethnography and narrative methodologies. She has a strong record of capacity building through supervision of academic Fellows (at pre-doctoral/doctoral/post-doctoral level), primary and secondary care clinicians, allied health professionals, social scientists, and anthropologists. She leads the APOLLO group, with 2-hour weekly meetings in which members collaborate to provide peer support, encouragement and constructive feedback on work-in-progress.

WIPH currently hosts 130 postgraduate research students. This includes two Wellcome-funded Doctoral Training Programmes (Health Data in Practice (HDIP)); Primary Care Clinicians). Prof. Swinglehurst is co-director of the HDIP programme and Dr Fudge is an Associate Director. The Institute also recruits doctoral students via the Trials Methodology Research Partnership DTP, the School for Primary Care Research PhD studentship scheme, the London Interdisciplinary Social Sciences DTP, the City of London Cancer Centre PhD studentships, and other national and international schemes. Lotte will therefore be well-supported by a network of peers and mentors. Academic support for WIPH PGRs is led by our excellent Graduate Studies team (Dr Fudge is Deputy Director) and our Graduate School offers induction and ongoing support including personal development workshops,

wellbeing and counselling, travel awards, training events.

WIPH provides regular opportunities for intellectual discussion, including the Early Careers Research Group, whiteboard sessions, seminars, the 'Curiosity Workshop' (qualitative researchers discussing work-in-progress) and the 'Thinking Between the Lines' critical theory group. Students can also present work at the annual WIPH graduate studies event and the Faculty William Harvey Day.

The QMUL Centre for Academic and Professional Development provides a comprehensive programme of short courses and bespoke support. These include writing and presentation skills, personal organisation, academic skills, career planning, and media training. The Centre for Public Engagement also provides funding to support public engagement and hosts the annual 'Festival of Communities' event in east London.

QMUL offers an unparalleled environment for this Fellowship. QMUL's research outputs, impact performance and research environment are all highly competitive as assessed in the 2021 Research Excellence Framework. We were ranked 7th in the UK for quality. Within the Faculty of Medicine and Dentistry (FMD) in Clinical Medicine 87% of outputs were ranked 3*-4*, 97% of impact statements were ranked 3*-4*, and 100% of the environment was ranked 3*- 4*. In Public Health, Health Services and Primary Care, 89% of outputs were ranked 3*-4*, with 100% of outputs ranked 3*-4* in both environment and impact case studies.

QMUL is one of the most ethnically diverse research-led universities in the world. Commitment to equality, diversity and inclusion (EDI) is embedded into all day-to-day and strategic decision-making within the Faculty, reflected in our research aims ('*Every life is precious'*). This work is overseen by six staff leads, who coordinate a network of academic and professional services staff with recognised roles within each institute. We proactively embed EDI into all meeting structures, policies and procedures. Faculty and institute-level leadership committees include EDI representation, and institute EDI reps ensure that faculty-level initiatives result in meaningful outcomes as well as developing and leading local change. FMD was the first QMUL faculty to receive a SWAN Bronze award (2008) and recently achieved Gold (2024).

FMD has a Flexible Research Fund scheme through which 20% of overheads earned on any FEC grant/award is returned to the investigator. Lotte would be able to use this for approved research-related activity (e.g. training, conference attendance, organising a symposium).

Developing our PhD students is pivotal to our contribution to research and education locally, nationally and internationally. QMUL would be delighted to host Lotte with this prestigious Award. We wish her every success.

9. Detailed Budget

The finance section should provide a breakdown of costs associated with undertaking the research as described in the proposal.

1

Staff type	Lead Applicant

Name	Lotte Elton

Employing organisation	Queen Mary University of London

Grade	Clinical Academic (C12)

Starting point on scale	20

Increment Date	01/10/2026

Justification of Costs

Staff costs

At the time of taking up the award I will be a fully-qualified GP (CCT date August 2025). In discussion with the finance team I have been placed on the QMUL paygrade (C12, spinal point 20) which most closely matches point 1 on the BMA pre-2003 consultant payscale (https://www.bma.org.uk/pay-and-contracts/pay/consultants-pay-scales/pay-scales-for-consultants-in-england). The salary requested includes London weighting (as I will be based in London throughout the fellowship) and the QMUL apprenticeship levy (included with London weighting in "other allowances").

Travel and subsistence

The research will be based in London, where I live, so direct travel costs are minimal. I have included funding to travel to Bristol twice a year to meet my supervisor, Dr Watson, in person. Many of the courses included in the training plan are in London, so no travel costs are required for these. Where training courses are outside London (e.g. Bristol, Oxford, Essex) I have included funding for standard-class rail fares, taking into account that I may have to travel at peak times (e.g. early in the morning to save on accommodation costs the previous night). Where a course outside of London lasts for more than one day, I have included funding for accommodation in standard business hotels at a rate of £100/night. This is within the limit for accommodation set by QMUL. Where there are cheaper options for accommodation (e.g. at the University of Essex), I have adjusted accommodation costs accordingly. I have included funding for meals at £30/day where I will be staying overnight in hotels.

I have requested funding for a 6-week research visit to Australia as well as a 2-week visit to Norway to allow me to develop my collaboration with Dr Stefan Hjörleifsson. All costed travel is standard class at approximate current rates. In both Australia and Norway I would plan to stay in standard business accommodation with cooking facilities (e.g. an aparthotel) as this means I would not require further subsistence costs. As I would be staying for a longer duration, accommodation costs would be <£100/night.

Equipment

I have requested funding for a digital voice recorder (for recording ethnographic interviews) and a video camera (for recording consultations). If I am able to I will borrow these from QMUL, but the length of time I will be spending in the field means that it may not be practical to use shared university equipment. I will need an encrypted hard drive to store this data and have included funding for a second back-up hard drive to prevent loss of data if the

principal hard drive breaks or is lost. On the advice of my supervisors, who have done video-reflexive ethnography workshops before, I have included funding for a video projector. This can be used when showing video clips during video-reflexive ethnography workshops and avoids any potential difficulties using encrypted hard drives on NHS computers.

Consumables

These costs include funding for printing consent forms and information leaflets/flyers. These will be distributed in research sites to introduce the research as well as in local GP practices to share the research findings.

PPI

I have embedded robust Patient and Public Involvement throughout the proposal. Public contributors will play a significant role not only in steering the research plan but also in contributing to the findings and dissemination of the research. I have budgeted for three meetings per year with the PPI panel, including travel expenses, refreshments, and childcare/care expenses as needed. I have budgeted separately for PPI contributors to participate in the day-long WP4 workshops to design and disseminate research outputs.

Two members of the public with extensive PPI experience will co-chair the Project Advisory Group. Both co-chairs were keen to attend some of the planned events (e.g. the Forum Theatre workshops) and wanted to meet in-person if possible. I have therefore budgeted for one in-person meeting per year.

I have included a training fund for PPI contributors (including the PPI panel and the lay co-chairs) to access training relevant to the research. I anticipate that the lay co-chairs may attend management training (e.g. on chairing meetings), and that some of the PPI panel may train in Forum Theatre methods alongside me. The choice of training courses will be made in discussion with PPI contributors.

In order to recruit local participants for the Forum Theatre workshops, I have included funding for Social Action for Health, a community health charity in East London which has worked closely with QMUL researchers for several years. They usually charge approximately £2,000 per focus group (with translators) so the £5,000 fee should cover two Forum Theatre workshops. They have longstanding links within the local community, including with people not usually reached by research (e.g. those who do not speak English as a first language). I will liaise with Social Action for Health to ensure that the Forum Theatre workshops are as representative of the local community as possible.

Dissemination costs

The fees paid to actors and to Improbable (who will help in recruiting actors and can also help to publicise the resources) will ensure that the Forum Theatre workshops are conducted by trained professionals with experience in improvisation and participatory theatre. I have been advised by Improbable to cost in £4,000 to compensate them for their staff members' time. I have set actors' fees at £300/day, roughly in line with Equity (the actors' union) industry standards for independent productions.

I have budgeted £6,000 in total to allow for the development of the resources for patients and clinicians (with separate fees for actors/public contributors). The £6,000 will cover fees for filming and licensing the short video clips, post-production costs, and website editing/design fees. I have budgeted a reasonable amount for these costs to ensure that the resources look professional and are user-friendly.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	89317
Geographical Weighting (£)	2292
Other allowances (£)	485
Superannuation (£)	13283
Nat. Insurance (£)	12991
Current Annual Costs (£)	**118368**

Staff Type: (Lead Applicant)-Dr Lotte Elton
Type of cost: HEI
% Full-time on this research: 100.0
Total months on this research: 36.0
Justification of Cost: The QMUL JMRO have placed me on the QMUL payscale which most closely aligns to the 1st rung of the pre-2003 consultant payscale. I will be a fully-qualified GP at the time of taking up the award.

	Year 1	Year 2	Year 3	Total Cost (£)
Basic Salary	89317	95217	101253	285787
National Insurance	12991	13876	14782	41649
Superannuation	13283	14139	15014	42436
Geographical weighting / Other allowances (if applicable)	2750	2780	2810	8340

	Year 1	Year 2	Year 3	Total Cost (£)
Total HEI	94673	100810	107087	302570
Total NHS	0	0	0	0
Total Commercial	0	0	0	0
Total Other	0	0	0	0

Travel, Subsistence and Conference Fees

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Return flights to Sydney, Australia
Justification of Cost: Return flights for the research visit to the University of Sydney/University of Wollongong.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	1500	0	1500

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Accommodation costs for Australia reseach visit.
Justification of Cost: 6 weeks of accommodation in a standard business hotel with cooking facilities (4 weeks in Sydney, 2 weeks in Wollongong)

	Year 1	Year 2	Year 3	Total Cost (£)
	0	4000	0	4000

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Return flights to Bergen, Norway
Justification of Cost: Return flights for research visit to Dr Stefán Hjörleifsson/University of Bergen.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	250	0	250

Type of cost: HEI
Description: Subsistence
Conference: False

Item Description: Accommodation costs for research visit to University of Bergen.
Justification of Cost: Costs for a two-week stay.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	1000	0	1000

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: SAPC conference 2026
Justification of Cost: Registration fee and accommodation costs for SAPC 2026 conference.

	Year 1	Year 2	Year 3	Total Cost (£)
	400	0	0	400

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: Preventing Overdiagnosis 2027
Justification of Cost: Registration costs (£430) plus travel and accommodation - conference location not yet decided but likely to be outside the UK.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	900	0	900

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: SAPC conference 2027
Justification of Cost: Registration fee/accommodation for the SAPC 2027 conference.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	400	0	400

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: RCGP conference 2028
Justification of Cost: Registration fee and accommodation costs for RCGP conference 2028.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	400	400

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: WONCA conference 2028
Justification of Cost: Registration fee, travel and accommodation for WONCA conference 2028 (conference location undecided but likely to be outside the UK).

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	900	900

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Bristol for supervision
Justification of Cost: Twice-yearly day trips to Bristol for supervisory meetings with Dr Jessica Watson. 6x return rail fares at £80 each.

	Year 1	Year 2	Year 3	Total Cost (£)
	160	160	160	480

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Oxford for Advanced Qualitative Research Methods course
Justification of Cost: One return rail fare @ £60

	Year 1	Year 2	Year 3	Total Cost (£)
	60	0	0	60

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Subsistence for Advanced Qualitative Research Methods course (University of Oxford)
Justification of Cost: 5 nights of accommodation in Oxford (£100/night), plus £30 subsistence/day

	Year 1	Year 2	Year 3	Total Cost (£)
	650	0	0	650

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to University of Essex for Ethnography and Ethnographic Methods course
Justification of Cost: One return rail fare @ £40

	Year 1	Year 2	Year 3	Total Cost (£)
	40	0	0	40

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Subsistence for Ethnography and Ethnographic Methods course
Justification of Cost: 5 nights of subsidised accommodation at the University of Essex (£240) plus £30/day subsistence.

	Year 1	Year 2	Year 3	Total Cost (£)
	390	0	0	390

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Ghent for the Linguistic Ethnography module at University of Ghent.
Justification of Cost: Eurostar return fare to Ghent, £250

	Year 1	Year 2	Year 3	Total Cost (£)
	250	0	0	250

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Subsistence for Linguistic Ethnography module.
Justification of Cost: 5 days of accommodation plus £30/day subsistence.

	Year 1	Year 2	Year 3	Total Cost (£)
	650	0	0	650

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Oxford for Knowledge into Action short course.
Justification of Cost: Return rail fare to Oxford @ £60

	Year 1	Year 2	Year 3	Total Cost (£)

	0	60	0	60

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Subsistence for Knowledge into Action course, University of Oxford
Justification of Cost: 5 nights of accommodation (£100/night) plus £30/day subsistence.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	650	0	650

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Bristol for Science Communication Masterclass
Justification of Cost: Return rail fare to Bristol @ £80

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	80	80

Type of cost: HEI
Description: Subsistence
Conference: False
Item Description: Subsistence for Science Communication Masterclass
Justification of Cost: 4 nights of accommodation in Bristol

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	400	400

	Year 1	Year 2	Year 3	Total Cost (£)
Travel, Subsistence and Conference Fees Total	2600	8920	1940	13460

Equipment

Type of cost: HEI
VAT relief to be applied: False
Description: Digital voice recorder
Price excluding VAT £: 250
Expected life time: 36
Justification of Cost: To be used for recording ethnographic interviews as well as the VRE workshops.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	0	250	0	250

Type of cost: HEI
VAT relief to be applied: False
Description: Video camera
Price excluding VAT £: 500
Expected life time: 36
Justification of Cost: For video-recording consultations as part of WP2.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	0	500	0	500

Type of cost: HEI
VAT relief to be applied: False
Description: Encrypted hard drive x2

Price excluding VAT £: 400
Expected life time: 36
Justification of Cost: Two encrypted hard drives (one as a back-up) for storing audio- and video-recorded data.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	0	400	0	400

Type of cost: HEI
VAT relief to be applied: False
Description: Video projector
Price excluding VAT £: 100
Expected life time: 36
Justification of Cost: To project recordings for VRE workshops (to avoid issues with using encrypted hard drives on NHS computer systems, as advised by my supervisors).

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	0	100	0	100

	Year 1	Year 2	Year 3	Total Cost (£)
Equipment Total	0	1250	0	1250

Summary

	Travel (excl Conferences)	Travel (Conferences)	Equipment
Total HEI	8368	3000	1000
Total NHS	0	0	0
Total Commercial	0	0	0
Total Other	0	0	0

Consumables

Description: Information leaflets and flyers for disseminating findings
Type of cost: HEI
Please describe fully: Information leaflets and flyers for disseminating findings
Justification of Cost: Colour flyers x200

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	100	100

Description: Consent forms
Type of cost: HEI
Please describe fully: Consent forms
Justification of Cost: 200 black & white consent forms to be printed

	Year 1	Year 2	Year 3	Total Cost (£)
	0	50	0	50

	Year 1	Year 2	Year 3	Total Cost (£)
Consumables Total	0	50	100	150

Patient and Public Involvement

Description: PPI panel fees for WP4 resource development workshops
Type of cost: HEI
Please describe fully: PPI panel fees for WP4 resource development workshops
Justification of Cost: Two day-long workshops: 6 participants x £150 x 2

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	1800	1800

Description: Travel expenses for WP4 resource development workshops
Type of cost: HEI
Please describe fully: Travel expenses for WP4 resource development workshops
Justification of Cost: £10pp per workshop

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	120	120

Description: PPI panel payments for project meetings
Type of cost: HEI
Please describe fully: PPI panel payments for project meetings
Justification of Cost: 2-hour meetings three times a year, £25/h, 6 members, plus £25 pre-meeting reading.

	Year 1	Year 2	Year 3	Total Cost (£)
	1350	1350	1350	4050

Description: Travel expenses for project meetings
Type of cost: HEI
Please describe fully: Travel expenses for project meetings
Justification of Cost: £10pp per meeting

	Year 1	Year 2	Year 3	Total Cost (£)
	180	180	180	540

Description: Refreshments for PPI meetings
Type of cost: HEI
Please describe fully: Refreshments for PPI meetings
Justification of Cost: £10pp per meeting

	Year 1	Year 2	Year 3	Total Cost (£)
	180	180	180	540

Description: Training fund for PPI panel/lay chairs
Type of cost: HEI
Please describe fully: Training fund for PPI panel/lay chairs
Justification of Cost: To allow PPI panel members to attend relevant training e.g. Forum Theatre training. Lay chairs will attend relevant methods or chairing training.

	Year 1	Year 2	Year 3	Total Cost (£)
	1000	900	0	1900

Description: Lay chair payments for Project Advisory Group meetings
Type of cost: HEI
Please describe fully: Lay chair payments for Project Advisory Group meetings
Justification of Cost: £100 per co-chair per meeting (2h meeting + 2h prep). Two meetings per year.

	Year 1	Year 2	Year 3	Total Cost (£)
	400	400	400	1200

Description: Childcare/care expenses for PPI panel members
Type of cost: HEI

Please describe fully: Childcare/care expenses for PPI panel members
Justification of Cost: To cover PPI panel members' childcare or care expenses if required.

	Year 1	Year 2	Year 3	Total Cost (£)
	200	150	150	500

Description: Lay chair travel expenses
Type of cost: HEI
Please describe fully: Lay chair travel expenses
Justification of Cost: 1 in-person meeting/event per year, £100 per chair (both are based in the North of England)

	Year 1	Year 2	Year 3	Total Cost (£)
	200	200	200	600

	Year 1	Year 2	Year 3	Total Cost (£)
Patient and Public Involvement Total	3510	3360	4380	11250

Other Direct Costs

Description: Transcription of VRE workshop recordings
Type of cost: HEI
Please describe fully: Transcription of VRE workshop recordings
Justification of Cost: Transcription of VRE workshops: 6x 1-hour, at £2.20 per minute (transcription rate for focus group/multiple voices)

	Year 1	Year 2	Year 3	Total Cost (£)
	0	792	0	792

Description: Transcription of ethnographic interviews
Type of cost: HEI
Please describe fully: Transcription of ethnographic interviews
Justification of Cost: Transcription of ethnographic interview data: 500 minutes at £1.50/min.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	750	0	750

Description: Dissemination event
Type of cost: HEI
Please describe fully: Dissemination event
Justification of Cost: Public event at the end of the project. £500 for venue hire, £500 for catering for approx 75 attendees.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	1000	1000

Description: Forum Theatre workshops
Type of cost: HEI
Please describe fully: Forum Theatre workshops
Justification of Cost: Actors fees (£150 per actor, 3 actors, 2 events) = £900 Venue hire = £1000 Refreshments at £10pp (30 attendees, 2 events) = £600

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	2500	2500

Description: Social Action for Health fees
Type of cost: HEI
Please describe fully: Social Action for Health fees
Justification of Cost: Payment to Social Action for Health for supporting the Forum Theatre workshops and recruiting attendees from the local community.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	5000	5000

Description: Refreshments for VRE workshops in WP3
Type of cost: HEI
Please describe fully: Refreshments for VRE workshops in WP3
Justification of Cost: £10pp, 15 people, 6 workshops

	Year 1	Year 2	Year 3	Total Cost (£)
	0	900	0	900

Description: Pre- and post-Forum Theatre workshops
Type of cost: HEI
Please describe fully: Pre- and post-Forum Theatre workshops
Justification of Cost: 2 workshops (to develop the play and then to record short scenes for patient/clinician resources). Actors fees (3 actors, 2 days) = £1800 Refreshments (£20pp, 10 people) = £200 Room hire (2 days) = £1200

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	3200	3200

Description: Improbable fees
Type of cost: HEI
Please describe fully: Improbable fees
Justification of Cost: Payment to Improbable for partnership and recruiting actors.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	4000	4000

Description: Filming and licensing of videos for clinician/patient resource
Type of cost: HEI
Please describe fully: Filming and licensing of videos for clinician/patient resource
Justification of Cost: £2000 for filming costs, £2000 for licensing fees.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	4000	4000

Description: Post-production and editing fees for WP4 resources
Type of cost: HEI
Please describe fully: Post-production and editing fees for WP4 resources
Justification of Cost: Costs for post-production editing and website design for WP4 resources.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	2000	2000

Description: Translation costs for participant materials/consent forms
Type of cost: HEI
Please describe fully: Translation costs for participant materials/consent forms
Justification of Cost: Translation of participant materials and consent forms into 6 different languages = £100 x 6

	Year 1	Year 2	Year 3	Total Cost (£)
	0	600	0	600

Description: Telephone interpreter costs
Type of cost: HEI
Please describe fully: Telephone interpreter costs
Justification of Cost: For extra interpreting costs incurred when consenting patients (who speak a language other than English) to participate in the study. This may be given to the research sites as a contribution, or used directly to pay telephone interpreter services. (Rates approx 40p/min)

	Year 1	Year 2	Year 3	Total Cost (£)

	0	200	0	200

Description: Research costs
Type of cost: NHS
Please describe fully: Research costs
Justification of Cost: Research costs as calculated in the SoECAT - site research costs for WP2 and WP3, including GP staff time to attend WP3 workshops. These have been discussed and agreed with the Lead Network.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	5401	0	5401

	Year 1	Year 2	Year 3	Total Cost (£)
Other Direct Costs Total	0	8643	21700	30343

Training And Development

Type of cost: HEI
Description: Applicant PhD Tuition Fees
Item Description: PhD fees
Justification of Cost: QMUL PhD fees for 2024-2025

	Year 1	Year 2	Year 3	Total Cost (£)
	4786	4786	4786	14358

Type of cost: HEI
Description: Short Courses
Item Description: Advanced Qualitative Research Methods, University of Oxford
Justification of Cost: Qualitative research course at University of Oxford. https://www.conted.ox.ac.uk/courses/advanced-qualitative-research-methods#fees_container

	Year 1	Year 2	Year 3	Total Cost (£)
	3175	0	0	3175

Type of cost: HEI
Description: Short Courses
Item Description: Ethnography and Ethnographic Methods course
Justification of Cost: 5-day course at University of Essex Summer School. https://essexsummerschool.com/summer-school-facts/courses/ess-2025-course-list/2q-ethnography-and-ethnographic-methods/

	Year 1	Year 2	Year 3	Total Cost (£)
	1200	0	0	1200

Type of cost: HEI
Description: Short Courses
Item Description: Linguistic Ethnography module
Justification of Cost: 5-day course at University of Ghent, 300 Euro. https://www.mils.ugent.be/courses/module-8-ethnographic-methods/

	Year 1	Year 2	Year 3	Total Cost (£)
	250	0	0	250

Type of cost: HEI
Description: Short Courses
Item Description: Creative Methods in Qualitative Data Collection
Justification of Cost: Course run by the Social Research Association. https://the-sra.org.uk/SRA/Shared_Content/Events/Event_display.aspx?EventKey=CRM080623

	Year 1	Year 2	Year 3	Total Cost (£)
	0	220	0	220

Type of cost: HEI
Description: Short Courses
Item Description: Forum Theatre Intensive Training
Justification of Cost: Run by the School of Theatre of the Oppressed. https://www.stoplondon.co.uk/forum-theatre-intensive

	Year 1	Year 2	Year 3	Total Cost (£)
	0	850	0	850

Type of cost: HEI
Description: Workshops
Item Description: Science Communication Masterclass
Justification of Cost: Run by UWE Bristol. https://courses.uwe.ac.uk/Z51000095/science-communication-masterclass

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	400	400

Type of cost: HEI
Description: Workshops
Item Description: Emerging Clinical Leaders Programme
Justification of Cost: Run by The King's Fund. Cost is £2,260+VAT (20%) = £2712. https://www.kingsfund.org.uk/leadership-development/courses/emerging-clinical-leaders

	Year 1	Year 2	Year 3	Total Cost (£)
	0	2712	0	2712

Type of cost: HEI
Description: Short Courses
Item Description: Build a website with WordPress
Justification of Cost: 1-day course run by CityLit. https://www.citylit.ac.uk/courses/build-a-website-with-wordpress/cwsd16-2425

	Year 1	Year 2	Year 3	Total Cost (£)
	129	0	0	129

Type of cost: HEI
Description: Overseas Research Visits
Item Description: Visa cost for Australia research visit
Justification of Cost: Cost for the Temporary Activities (Research) 408 visa for the 6-week research visit to the University of Sydney/University of Wollongong.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	250	0	250

	Year 1	Year 2	Year 3	Total Cost (£)
Training And Development Total	9540	8818	5186	23544

Total Direct Costs

	Year 1	Year 2	Year 3	Total Cost (£)
Total Direct Costs	109181	128747	135029	372957

Indirect costs

HEI Indirect costs

Type of cost: Other Indirect Costs
Name of Higher Education Institute: Queen Mary, University of London
Justification of Cost: HEI indirect costs

	Year 1	Year 2	Year 3	Total Cost (£)
	67734	67734	67734	203202

Type of cost: Estates Charges
Name of Higher Education Institute: Queen Mary, University of London
Justification of Cost: Estate costs, including infrastructure technician costs.

	Year 1	Year 2	Year 3	Total Cost (£)
	22567	22567	22567	67701

	Year 1	Year 2	Year 3	Total Cost (£)
HEI Indirect costs Total	90301	90301	90301	270903

Commercial Indirect Costs
No costs have been added

Other Partner Organisation Indirect costs
No costs have been added

	Year 1	Year 2	Year 3	Total Cost (£)
Total Indirect Costs	72241	72241	72241	216723

Research Costs Requested From Funder

	Direct costs	Indirect costs/Overheads	Amount requested
Total Higher Education Institution Costs (80%)	341012	216723	557735
Total Higher Education Institution Costs (100%)s	26544	0	26544
Total Commercial Costs	0	0	0
Total Other Parther Organisation Costs	0	0	0
Total NHS Research Costs	5401	0	5401
Total Research Costs Requested From Funder	372957	216723	589680

Summary

	Year 1	Year 2	Year 3	Total Cost (£)
Salaries	**94673**	**100810**	**107087**	**302570**
Travel-subsistence-and-conference-fees	**2160**	**7396**	**1812**	**11368**
Equipment	**0**	**1000**	**0**	**1000**

Consumables	**0**	**40**	**80**	**120**
Patient-and-Public-Involvement	**2808**	**2688**	**3504**	**9000**
Other-Direct-Costs	**0**	**7995**	**17360**	**25355**
Training And Development	**9540**	**8818**	**5186**	**23544**
HEI-Indirect-costs	**72241**	**72241**	**72241**	**216723**
Commercial-Indirect-Costs	**0**	**0**	**0**	**0**
Other-Partner-Organisation-Indirect-costs	**0**	**0**	**0**	**0**
Total	**181422**	**200988**	**207270**	**589680**

NHS Support Costs

Cost Item:	Year 1	Year 2	Year 3	Total Cost (£)
NHS Support Costs from SoECAT	0	670	0	670.0

Justification:
NHS Support Costs from SoECAT: NHS Support Costs (e.g. set-up, consenting patients and GP staff), as laid out in the SoECAT. These costs have been agreed with the Lead Network.

	Year 1	Year 2	Year 3	Total Cost (£)
NHS Support Costs Total	0	670	0	670

Have you discussed and agreed these costs with the Lead Network?	Yes

NHS Treatment Costs
No costs have been added

Have you discussed and agreed the NHS costs with the Lead Trust?	Yes

Is the patient/service user care being provided different from the usual standard treatment for the condition?
No

Usual Treatment Costs
No costs have been added

Excess Treatment costs/(savings)

	Cost per patient(£)	Year 1	Year 2	Year 3	Total Cost (£)
Total NHS Excess Treatment costs/(savings)	0	0	0	0	0

	Total (£)
NHS Support Costs requested from networks	670
NHS Excess Treatment costs/savings	0

TOTAL NHS SUPPORT AND TREATMENT COSTS	670

Total Funding Required (including non-research costs)

	Total (£)
Total Research Costs Requested from Funder	589680
Total NHS (Support and Excess Treatment) Costs	670
Total Funding Required	590350

10. Uploads

Please upload the following documents, if necessary.

When uploading a document to your application, please ensure that it is in an appropriate file format. It is not possible to upload JPEG images, and with the exception of the SoECAT Form, which should be uploaded as a .csv file or Excel spreadsheet, all files should be uploaded in Adobe PDF or Word DOC/DOCX format. Please refer to the Guidance Notes for further information on the attachments required.

(References - NIHR References.docx) is included as an appendix within this file.

(Research Timetable - GANTT chart for PhD.pdf) is included as an appendix within this file.

(SoECAT Template - Funder Export_1011538_SoECAT_The UNITE study.xlsx) is included as an appendix within this file.

11. Participants and Signatory

Participants
New participants must confirm their participation on your application before you will be able to select the 'SUBMIT' option. Please see the Submission Process Flow Diagram and the Applicant Guidance Notes for further information.

You are required to supply the names and email addresses (if not already registered with this online application system) of each required participant. Everyone **newly** named in this section will be acting as a 'participant' to your application and will need to agree to be part of this application.

By confirming participation, participants are acknowledging their involvement and input into this application, and agreeing to be involved in the proposed fellowship. You must ensure all **new** participants are happy for your application to be submitted before submitting it on the online system.

It is permissible for a single individual to serve as a signatory and a participant, but they will need to be added separately under each heading.

Details of the individual who will be required to approve your application after submission should be entered in the 'Participants and Signatory' section. Please note that it is acceptable for a Head of Department to also act as a research support role. If this is the case, they will need to be included in this section for both roles.

Participants must confirm their participation on your application before you will be able to press the submit button. Please see the 'Application Submission Process Flow Diagram' in the guidance notes for further information.

Doctoral Primary Supervisor

Doctoral Primary Supervisor Participant

Title	Professor
First Name	Deborah
Last Name	Swinglehurst
Position	

Supervisor

Supervisor Participant

Title	Dr
First Name	Nina
Last Name	Fudge
Position	Lecturer

Supervisor Participant

Title	Dr
First Name	Jessica
Last Name	Watson

Position	GPST4 Academic Clinical Fellow

Administrative Authority or Finance Officer

Administrative Authority or FO Participant

Title	Mrs
First Name	Coleen
Last Name	Colechin
Position	Operations Manager

Signatory
Participants must complete their actions on your application before submission; the signatory must also approve your application after you select the 'submit' option but BEFORE the application submission deadline.

You are required to supply the name and email address (if not already registered on the ARAMIS application system) of the individuals who will be 'signing off' your application. Once their contact details have been entered below, these individuals will be invited to log in and confirm their participation in your application.

The signatory must ALSO approve the application AFTER you have selected the 'SUBMIT' option but BEFORE the application submission deadline. Please see the Submission Process Flow Diagram and Applicant Guidance Notes for further information.

The signatory approval will result in the application being fully submitted to the NIHR. All parties (applicant, participants and signatory) will be notified of this via an automated system-generated email.
NIHR will not accept any applications unless fully approved by your signatory before the 1:00 pm deadline on Thursday 16th January 2025.

Host Organisation - Heads of Department

Head of Department or Senior Manager

Title	Professor
First Name	claudia
Last Name	cooper
Position	Professor of Psychological Medicine

12. Acknowledgement, Review and Submit

Conflict checks: Please declare any conflicts or potential conflicts of interest that you may have in undertaking this research, including any relevant, non-personal & commercial interest that could be perceived as a conflict of interest.
I have no conflicts of interest to declare.

By completing this form, the programme reserves the right to share, in confidence, details of your application with other parts of the NIHR and with the Department of Health and Social Care.

Agreement to Terms and Conditions: I confirm that I have secured all necessary licences and ethical approvals in relation to the research and will abide by the terms of those licences and approvals in carrying out the research. I confirm that I have read the NIHR Carbon Reduction Guidelines and, where possible, have taken steps to reduce the carbon emissions generated by this research. I confirm that all material within this application, with the exception of sections that other participants are required to complete, is my own work and I have taken steps to ensure that no parts of the application have been inappropriately plagiarised. I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time lay down according to the NIHR Research Governance Framework. If you are applying for an NIHR award which comes with partnership funding from another organisation (e.g. a charity or industry partner) your information (including your NIHR Application Form and subsequent progress/annual reports) may be shared with the partner organisation as part of the review process and ongoing monitoring of the award. In completing this, I declare that, to the best of my knowledge, the information provided in this application, is true, accurate and complete. I have read the Guidance Notes and agree to accept the process by which an application is assessed and agree to abide by the conditions under which an award may be granted.
☒Confirmed

Appendices

1. NIHR References.docx
2. GANTT chart for PhD.pdf
3. Funder Export_1011538_SoECAT_The UNITE study.xlsx

Appendix #1: NIHR References.docx

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Appendix #2: GANTT chart for PhD.pdf

Research project timetable

		Year 1												Year 2												Year 3											
		Oct-25	Nov-25	Dec-25	Jan-26	Feb-26	Mar-26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27	Apr-27	May-27	Jun-27	Jul-27	Aug-27	Sep-27	Oct-27	Nov-27	Dec-27	Jan-28	Feb-28	Mar-28	Apr-28	May-28	Jun-28	Jul-28	Aug-28	Sep-28
Project management																																					
	Study, training and development set-up	■	■	■																																	
	Upgrade progression review									■																											
QMUL	Progress reviews										■									■									■								
	PPI panel meetings				■				■				■				■				■				■				■				■			■	
External	Advisory board meetings						■					■					■					■					■					■					
Project plan																																					
WP1	Meta-ethnography	■	■	■	■	■	■	■	■	■	■																										
	Ethics application		■	■	■	■	■	■	■	■																											
	Recruitment of GP research sites										■	■	■																								
	Ethnographic observation													■	■	■	■	■	■																		
WP2	Recording of consultations																■	■	■																		
	Video-reflexive ethnography workshops																	■	■	■	■																
WP3	Transcription of video-reflexive ethnography workshops																		■	■	■																
	Forum Theatre play development																										■										
	Forum Theatre workshops																											■	■								
	Resource development																													■	■	■	■				
WP4	Analysis of data from WPs 2, 3 and 4													■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■				
Training and development																																					
	Dr Hjörleifsson/Choosing Wisely, University of Bergen *(2 weeks)*																					■															
Visits	University of Sydney/University of Wollongong *(6 weeks)*																								■	■											
	Advanced Qualitative Methods (University of Oxford) *(8 weeks part-time)*							■																													
	Literature Searching for Systematic Reviews (QMUL) *(half-day)*	■																																			
	Overview of Qualitative Synthesis Methods (Uni. Bristol) *(1 day)*			■																																	
	Ethnographic and Ethnographic Methods (Uni. Essex) *(5 days)*										■																										
	Linguistic Ethnography (Ghent University) *(5 days)*										■																										
	Creative Methods in Qualitative Research (SRA) *(2 days)*																					■															
Methods	Forum Theatre Intensive Training *(5 days)*														■																						
	Co-design, Co-production and PAR (Uni. Bristol) *(1 day)*																			■																	
	Inclusivity in Research Design (Uni. Bristol) *(1 day)*			■																																	
	Hands on Engagement: Taking Research to Events (QMUL) *(1 day)*																				■																
	Knowledge into Action (University of Oxford) (*8 weeks part-time)*																			■																	
	Science Communication Masterclass (UWE Bristol) *(4 days)*																										■										
Dissemination	WordPress website design (CityLit) *(1 day)*				■																																
Development	Emerging Clinical Leaders (King's Fund) *(6 days over 4 months)*																								■	■	■	■	■								
Dissemination																																					
	Paper 1: Meta-ethnography										■	■	■	■																							
	Paper 2: Ethnographic observation/ close analysis of consultations																			■	■	■	■														
	Paper 3: Video-reflexive ethnography																								■	■	■	■									
Papers	Paper 4: Forum Theatre/co-production																													■	■	■	■				
	SAPC 2026										■																										
	SAPC 2027																						■														
	Preventing Overdiagnosis 2027																							■													
	RCGP 2027																										■										
Conferences	WONCA 2028																																				■
	Develop project website				■	■																															
	Blog posts						■				■				■				■				■				■				■				■		
	Thinking in Between podcast episode																																■	■			
	Stakeholder event																																		■		
Other outputs	Writing thesis								■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■	■

Appendix #3: Funder Export_1011538_SoECAT_The UNITE study.xlsx